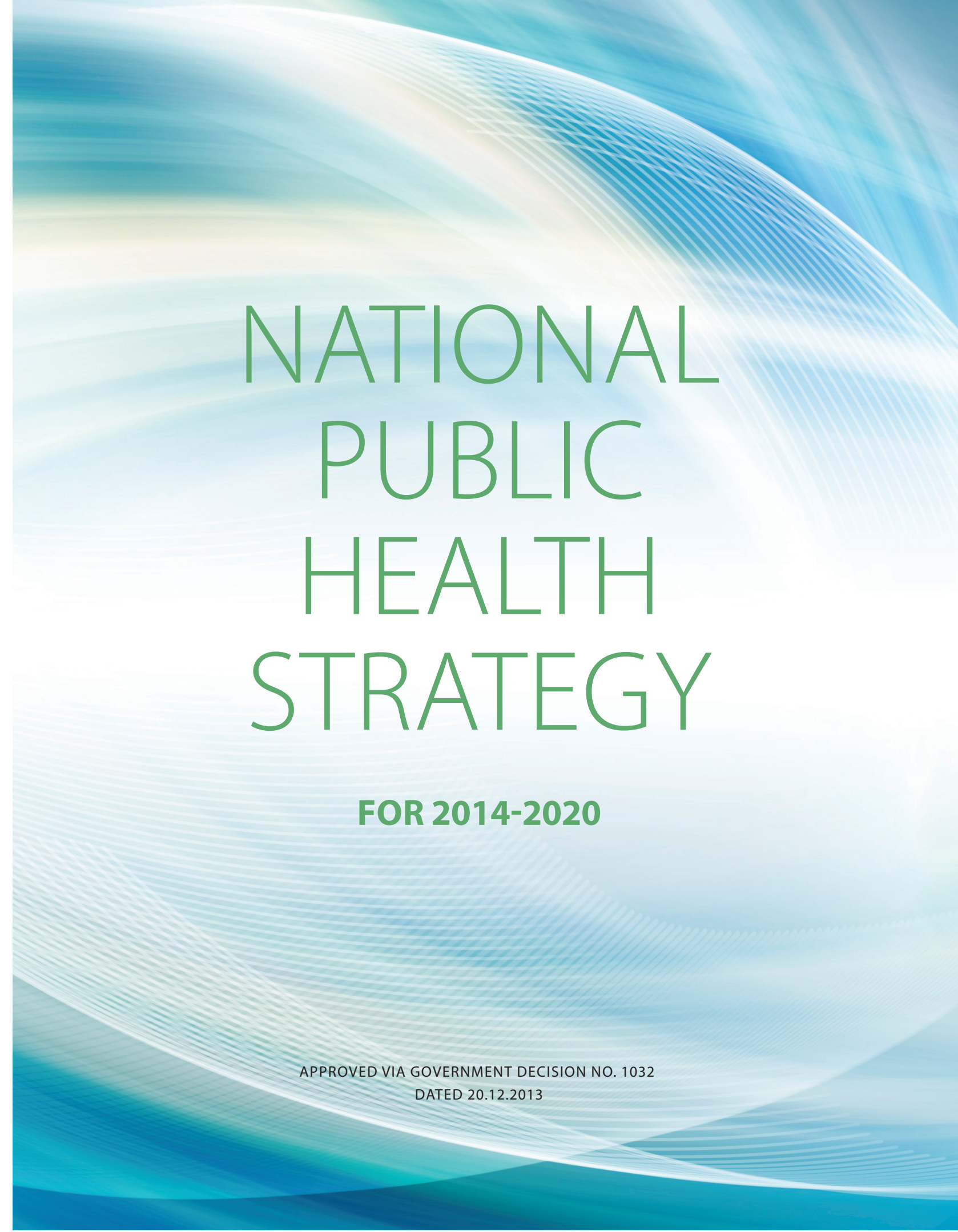




NATIONAL PUBLIC HEALTH STRATEGY

FOR 2014-2020

APPROVED VIA GOVERNMENT DECISION NO. 1032
DATED 20.12.2013



I. INTRODUCTION

1. Public Health represents a set of scientific-practical, legislative, organizational, administrative and other measures, the significance of which has evolved over the time from the strict focus on control of infectious diseases, environmental health, health protection, and health education towards a comprehensive approach of population health status.
2. The National Public Health Strategy 2020 (hereinafter referred to as the *Strategy*) is developed to support the implementation of the Republic of Moldova Government's Activity Programme "European Integration: Freedom, Democracy, Wellbeing 2013-2014", the National Development Strategy "Moldova – 2020", the Health System Development Strategy for 2008-2017, the National Health Policy (2007).
3. The general principles which served as basis for developing the Strategy are formulated in different international and national documents, which tackle the area of public health, such as the WHO Framework Policy "Health 2020", which aims to support the interactions between the Government and the society so as to improve significantly the population health and wellbeing, to reduce inequalities in health area, and to enhance public health. On a priority basis, the Strategy shall focus on carrying out the Post-2014 Action Program of the International Conference on Population and Development and the Post-2015 Sustainable Development Agenda.
4. This Strategy describes the current situation in the area, defines the general and specific objectives, reveals the necessary actions and measures, and sets clear responsibilities and implementation deadlines for all the stakeholders involved in activities related to public health. The crosscutting issues include human rights, reproductive rights, and gender equality, inclusive partnerships and humanitarian assistance. The Strategy will develop the national capacities for ensuring fair access to integrated health services.
5. The responsibility for regulating, coordinating, and controlling the functioning of the public health system stays with the Ministry of Health and its specialized structures, which are organized by duty and responsibility criteria at the national and territorial levels.

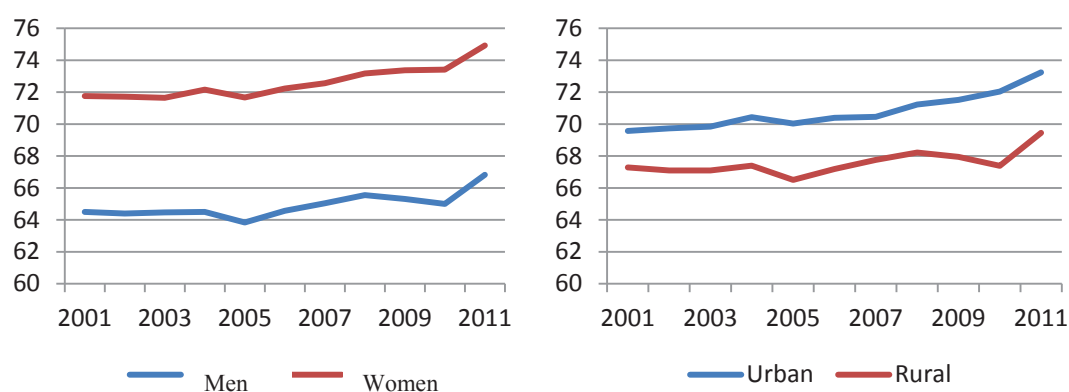
II. CURRENT SITUATION BACKGROUND

Section 1. Population health status

6. **Life expectancy at birth** in the Republic of Moldova is one of the lowest in the European Region. In 2010, Moldova succeeded to outrun only Russia and Kazakhstan, registering a life expectancy at birth rate of 69.13 years (Health for all database, WHO Europe, 2012). Over the last year (2011) a slight improvement was achieved, registering an average of 70.88 years (for men – 66.82 and for women – 74.92 years). A substantial difference is registered for life expectancy in urban and rural areas, 73.24 and 69.45 years, respectively (Fig.1).

7. The Republic of Moldova has entered a **deep demographic transition** period, inducing pressure on the public health system. The total fertility rate is 1.3 born children/woman. Together with the increased migration incidence and the increase of life expectancy, the low fertility rate feeds the change of population structure. The population ages rapidly.

Figure 1. Life expectancy at birth by sex and area of residence, for 2001-2011



Source: National Bureau of Statistics (hereinafter referred to as the NBS), 2012

8. The Republic of Moldova registers high values for **population general mortality**, being ranked among the countries with the highest mortality rate in the European Region. In 2012, the general mortality rate accounted for 1111.4 cases per 100 000 population of all ages (National Center for Health Management (hereinafter referred to as NCHM), Medical Statistical Yearbook, 2013). Non-communicable diseases accounted for 88% of the total number of deaths registered in 2011 (Tab. 1). The mortality rates caused by non-communicable diseases register differences depending on geographic zones, areas of residence, and sex. Most of deaths are caused by circulatory system diseases (57.5%); tumors are on the second place (14.5% deaths of the total number); followed by – digestive system diseases (9.1%) on the third place; injuries, poisonings, and other consequences of external causes (7.8%) on the fourth place; and respiratory system diseases (4.8%) on the fifth place. A specific characteristic of the mortality structure for the population of the Republic of Moldova is a rather high mortality rate induced by hepatic cirrhosis (8% out of the 9.1% of deaths determined by the digestive system diseases), without any sex differences.

9. Another national feature is the **high mortality rate among the working-age population**. A total of 456.2 deaths were registered per 100 thousand inhabitants in 2012 among the population of working-age (men – 696.4 and women – 201.1) (NCHM, Medical Statistical Yearbook 2013), accounting in average for 44% of the total number of deaths (men's share of the total is 60% and women's share is 23%). The mortality structure for the working-age population reveals the following distribution: the circulatory system diseases are on the first place (24.7%), followed by injuries, poisoning and other consequences of external causes (21.7%), tumors (18.3%), digestive system diseases (18.3%), and respiratory system diseases (6.7%). Cumulatively, these five categories account for about 90% of all causes of deaths. From area of residence prospective, the overall mortality rate is significantly higher in rural area than in urban.

10. Over the last eight years, the **infant mortality rate registered a decreasing trend, accounting for 9.8 per 1000 live births** in 2012 (Tab. 2). This rate is lower in comparison with the CIS countries, but higher than the rate registered in the EU-27 Member States (Health for all database, WHO Europe, 2012).

11. **Maternal mortality rate** also tends to decrease, registering nevertheless two increase picks in 2008 and 2010 (Tab. 2). In 2010, maternal mortality was identified as an indirect obstetrical risk in half of all registered deaths, while 5 of them were reported as being induced by the pandemic flu. The twofold difference between the maternal mortality rate registered in urban and rural areas may be partially explained by health services availability (and quality) and unequal distribution of resources.

12. **The incidence of circulatory system diseases** is relatively low: a number of 194.2 cases per 10 thousand inhabitants were registered 2012, the most frequent being the incidence of hypertension – 124.0 cases per 10 thousand inhabitants (NCHM, Medical Statistical Yearbook 2012). The prevalence of the circulatory system diseases is rather high, registering 1417.4 cases per 10 thousand inhabitants (17.5% of the general prevalence), ranking on the second place among the categories of diseases (NCHM, Medical Statistical Yearbook 2012).

Table 1. Some health indicators, 2003–2011

	2003	2004	2005	2006	2007	2008	2009	2010	2011
Non-communicable diseases, standardized mortality rate per 100 000 population, all ages									
Circulatory system diseases	857.5	805.2	858.4	786.4	849.3	716.5	715.2	731.1	664.7
Malignant tumors	155.5	158.4	161.2	166.2	161.9	164.7	168.4	165.3	163.1
Respiratory system diseases	96.7	81.2	92.6	83.6	78.8	74.3	69.6	718	55.8
Digestive system diseases	129.4	130.9	143.1	134.2	127.4	118.5	120.7	126.2	102.6
Liver chronic diseases and hepatic cirrhosis	103.8	104.7	114.2	107.1	103.0	96.7	96.1	102.7	81.4
Diabetes	11.7	9.9	11.6	12.1	11.4	10.1	9.6	10.7	8.9
Other external death causes, standardized mortality rate per 100 000 population, all ages									
Road accidents	15.2	15.5	14.4	13.6	15.9	15.2	12.9	12.4	12.8
Traumas and poisonings	110.6	107.6	113.8	109.0	103.7	100.2	97.2	103.0	85.0
Alcohol use related causes	223.0	221.6	239.0	227.1	219.2	207.4	205.2	217.9	175.3
Smoking related causes	930.8	862.4	917.2	838.7	791.9	755.1	748.2	764.4	689.0

Source: Health for all database, WHO Regional Office for Europe, 2012.

Table 2. Infant and maternal mortality rates, 2004–2012

	2004	2005	2006	2007	2008	2009	2010	2011	2012
Infant mortality rate, per 1000 live births	12.2	12.4	11.8	11.3	12.2	12.1	11.7	10.9	9.8
Maternal mortality, per 100 thousand live births	23.5	18.6	16.0	15.8	38.4	17.2	44.5	15.3	30.4

Source: NCHM, Medical Statistical Yearbook 2004–2011.

13. Tumors register a high mortality rate (14.5% deaths of the total number), but the incidence and prevalence of tumors are low, accounting in 2012 for 42.0 and 211.7 cases per 10 thousand inhabitants, respectively (NCHM, Medical Statistical yearbook, 2012) – speaking for a rather low addressability level, an insufficient early diagnosis, and inefficient treatment.

14. The incidence of digestive system diseases in 2012 accounted for 224.1 cases per 10 thousand inhabitants or 7.4% of the total number of contacts, hence being ranked on the third place. The prevalence of digestive system diseases accounted for 953.8 or 12.8%, being ranked on the third place among the prevalence of all morbid conditions (NCHM, Medical Statistical Yearbook, 2012).

15. The incidence of injuries, poisonings, and other consequences of external factors accounted in 2012 for 361.0 cases per 10 thousand inhabitants or 9.7% of the total number of contacts, thus being ranked on the fourth place. The prevalence accounted for 364.2 cases per 10 thousand inhabitants or 4.6% of the prevalence of all morbid conditions (NCHM, Medical Statistical Yearbook, 2012).

16. The incidence of respiratory system diseases is very high (in 2012 – 1023.6 per 10 thousand inhabitants), being ranked on the first place (35.8%) of all the categories of diseases (NCHM, Medical Statistical Yearbook, 2012). Most of visits were conditioned by pneumonias (72.3 cases per 10 thousand inhabitants).

17. The incidence of diabetes is relatively high in the Republic of Moldova and registers increasing trends. In 2012, the

incidence of diabetes registered 25.2 cases per 10 thousand inhabitants (NCHM, Medical Statistical Yearbook 2012). The prevalence of diabetes has also registered increasing trends, from 145.5 cases per 10 thousand inhabitants in 2008 up to 201.1 cases in 2012, of which 37.1 cases are insulin-dependent (NCHM, Medical Statistical Yearbook 2012).

18. The burden of **communicable infectious diseases and parasite diseases** in the structure of population general morbidity accounts for more than 15%, registering an incidence of 231.9 cases per 10 thousand population (NCHM, Medical Statistical Yearbook 2012). The general morbidity of infectious and parasite diseases is mainly determined by acute viral respiratory infections (AVRI, flu and pneumonia), parasite diseases (helminthiasis), acute diarrheal diseases (ADD), which account for 90% of the total number of registered infectious diseases. ADD morbidity continues to be determined by affections registered in children aged 0-17 years old (76%), including children aged 3-6 years old who attend pre-school institutions. Helminthiasis has an important place in the general infectious morbidity structure, registering annually about ~17 thousand cases, of which 85% of children aged up to 17 years old. Tuberculosis and HIV/AIDS also remain to be among the priority problems for the Republic of Moldova.

19. The DALY (disability-adjusted life year) indicator measured by WHO in 2002 presents **10 priority risk factors**, which contribute to the burden of diseases in different groups of countries (WHO, 2006). The main risk factors for the Republic of Moldova in men's case (53.4% of the total DALY) are the following: harmful use of alcohol, smoking and high blood pressure; and three main risk factors are registered in women's case: high blood pressure, high level of cholesterol, and excessive use of alcohol, accounting for 41% of the total DALY.

20. A rather high share of adult population of the Republic of Moldova **consumes alcohol**. The results of the Demography and Health Survey carried out in 2005 in the Republic of Moldova (the National Scientific-Practical Center of Preventive Medicine (NSPCPM), Moldova and ORC Macro, 2006) reveals that 59% of women and 81% of men used at least one alcoholic beverage during the month before the survey. A total of 17% of men reported daily or almost daily use of alcohol and 41% consumed alcohol at least once per week.

21. The results of the Demography and Health Survey (NSPCPM (Moldova) and ORC Macro, 2006) reveal significant differences in **smoking prevalence** among women and men: 51% of men and 7% of women reported that they smoke. Smoking is more frequent among rural men (53%) than urban men (49%). Nevertheless the situation is reversed in women's case: 2% of rural women and 14% of urban women smoke.

22. Overweight and obesity, as well as their associated diseases, represent an important public health problem. About 50% of the adult population of the Republic of Moldova (18 years old +) are overweighted or obese. The share of obese persons accounts for 15% of this number, and it is higher in urban area (16.3%) as compared to rural area (14.7%); registering also a higher share among women (17.1%) than among men (13.3%). Obesity increases proportionally to the age, from 1% in the age group 18-24 years old up to 25% in the age group 45-65 years old, and afterwards it decreases.

23. Sedentary lifestyle is a characteristic feature for 59.2% of the population aged 15 years old and over. Taking into consideration the area of residence, the share of persons with a sedentary lifestyle is higher in urban area as compared to rural area (62.0% versus 57.1%). Men practice sport activities more frequently than women. The intense sport activities are practiced, in average, by almost 1.9% of population, usually men, and at a larger extent by the age group of 15-24 years old (19.3%). As the age grows, the share of persons practicing intense or moderate sport activities goes down.

24. Consumption of products rich in fats, salt, and sugar. The excessive consumption of these products in combination with the sedentary lifestyle leads to obesity, high blood pressure, and non-communicable diseases' outbreak. The average daily dietary consumption of energy per capita in the Republic of Moldova is higher than the dietary need of energy (estimated at 2141 kcal/per capita/per day), accounting in 2009 for 2707 kcal/per capita/per day. The highest share of dietary energy (65%) registered in 2009 originated in carbohydrates, followed by lipids (25%) and proteins (10%).

25. Deficiency of micronutrients. A limited number of population-based studies were carried out to determine how widely the micronutrients' deficiency is spread. Currently data exist only for iron deficiency (using anemia as an indicator) and iodine deficiency, which have revealed a high level for such deficiencies' spread. The Demography and Health Study from 2005 has determined that 52.3% of children aged 6-11 months old, 32.2% under-five years old children, and 40.4% of women who reported that they are pregnant, as well as 27.8% women of fertile age had anemia. The disorders induced by iodine deficit are recognized as a public health problem in the Republic of Moldova. *In this area were undertaken a number of measures to decrease the spread of iodine deficiency.* The study carried out in 2012 has identified that the median excretion of iodine through urine in care of pupils has accounted for 204 µg/l at the national level and it is increased as compared to the previous years (78, 4 µg/l in 1996), being within the limits recommended by WHO for the optimal intake of iodine (100-300 µg/l).

26. There are many problems related to **occupational health**. Over the last five years, a number of 87 cases of professional diseases were registered (2008-2012) with 92 affected persons, accounting for an average of 1.65 per 100 thousand employees. The most frequent forms of registered diseases were conditioned by vibration (48.9%), pathogenic agents (16.3%), toxic chemical substances (15.2%), agro-chemicals (13.0%) powder (1.1%), and other factors (4.3%).

Section 2. Analysis of priority intervention areas.

Population health surveillance

27. One of the basic functions of the State Public Health Surveillance Service mentioned in the Law No. 10-XVI dated 03.02.2009 on State Public Health Surveillance Service (SPHSS) is to study and assess comprehensively health problems and to develop adequate measures of prevention and control.

28. The collection of data regarding the population health status and demographic indicators is done by a number of institutions. The NBS is the central statistical authority subordinated to the Government. It is responsible for collecting, verifying, and analyzing demographic, economic, social, and other type of data, maintaining the statistical database and keeping it up to date; carrying out periodic population censuses and other surveys, as well as developing and publishing reports. The health protection indicators are submitted to the NBS by the Ministry of Health, which, at its turn, has two central institutions: the NCHM and the National Public Health Center (NPHC) which collect data on population health status.

29. The NCHM is responsible for collecting, processing, and assessing statistical data about population health (including morbidity, trauma, disability, and other) and medical institutions' activity, including in-

stitutions' financing and staffing. The NPHC is responsible also for collecting, processing, and assessing the data on communicable diseases, public health risks and emergencies, as well as health determinants: occupational, environmental, and behavioral. The NPHC reports the data on communicable disease and risk factors to the NCHM, while the NCHM sends the NPHC the data on non-communicable diseases. It is important to mention that the databases and the information systems of these institutions are not integrated, and the data are exchanged in hardcopies. The existing data are not standardized and classified according to the same criteria, and their quality is verified very rarely.

30. The medical institutions have data on morbidity of reference population, but this information in majority of cases is on hardcopies. The data are reported by the medical institutions to the NCHM also on hardcopies, except for the communicable diseases, which are monitored in real time, and the NPHC manages the surveillance system for these diseases. The respective system also includes data on public health emergencies. The data flow includes the supply (in electronic format or in hardcopies) of primary data about the cases of infectious diseases, reported by the primary, emergency, and specialized (infectious diseases doctors) healthcare and hospitals – in case of nosocomial infections, to the territorial Public Health Center (PHC). The PHC collects, totalizes, and analyzes the received data with a certain periodicity – daily, weekly, monthly, annually – by filling in reports: operative – in case of epidemics; weekly – the weekly surveillance system (ADD, AVRI) and sentinel (ILI, ARI, SARI); monthly – routine surveillance of infectious morbidity. The case definitions developed based on the Decision No. 2119/98/EC of the European Parliament and of the Council setting up a network for the epidemiological surveillance and control of communicable diseases in the Community are used to standardize the data collection process. The PHC collects and analyzes data on parasitosis, vaccinations, including coverage with vaccination.

31. Some specialized medical institutions have established and manage national registers for some groups of diseases, but they are not integrated with other databases and all of them are reported to the Ministry of Health and central surveillance institutions on hardcopies.

32. Periodically, with the financial support of international organizations, population-based studies are carried out (DHS, MICS 4, survey on smoking among teenagers, etc.). All of them allow accumulating deep information in the area of public health.

33. The national nutritional surveillance system was established in 2012. Initially it was implemented in three pilot regions, and after the assessment which will be carried out in the first quarter of 2014, it will be suggested to extend it at the national level.

34. At the district level, the PHC assesses annually the population health status with the help of district medical institutions, but all of them are narrative and not analytical.

Section 3. Preparing, planning, monitoring and responding to public health risks and emergencies

35. At the national level, in compliance with the Government Decision No. 1340/2001, the Commission for Exceptional Situations of the Republic of Moldova Government, represented by different ministries and

departments, is responsible for preventing the planning and management actions in exceptional situations of natural calamities, high-scale accidents, catastrophes, fires, epidemics, epizootics, etc. The Civil Protection and Exceptional Situations Service (CPESS) is the national authority responsible for implementing policies in the area of exceptional situations and developing the National Multi-sectorial Plan for Civil Protection including components from the sector plans of the ministries and governmental agencies. In case of health events and emergencies, the National Extraordinary Commission for Public Health is responsible for the integrated approach to public health threats and emergencies, application of prevention and management measures, multi-sector mobilization, and coordination of activities. The Law No. 10-XVI/2009 on State Surveillance of Public Health delimits the functions and duties by health system levels in relation to preventing, ensuring an adequate preparedness level, management of public health emergencies, detection, evaluation, and notification of public health risks and emergencies, declaration/cancelling of public health emergency.

36. The legal framework in the area of public health emergencies is not completely adjusted to the requirements of the European Union. No preparedness and response plans are developed for all the cases of public health emergencies, including the protocols and guidelines for planning, preparing, training in assessment of risks, needs, and vulnerabilities, application of case definitions, management, communication, for preventing and intervening in cases of accidental or intentional deliberation of biologic agents.

37. The SPHSS keeps a national system for surveillance of public health events, and the NPHC is appointed as the National Focal Point for the ISR (2005).

38. The Ministry of Health and the CPESS assess continuously the preparedness level at the institutional, district, and national levels, and train different groups of specialists from health and other sectors (Customs, Border Police, Sanitary-Veterinary Service, etc.). In spite of this fact, insufficient financial resources are allocated for ensuring the continuity of training and revising/updating periodically the training materials.

39. The SPHSS has a monitoring system for environmental and occupational factors. The NPHC develops procedures for identifying the risk exposure and assessing the impact on public health for short and long terms. The SPHSS updates periodically the list of industrial institutions and the register of chemical substances used in production.

40. The results of the routine investigations fulfilled within the systems of epidemiological surveillance, monitoring of risk factors and health determinants, which are carried out by the SPHSS laboratories, serve as basis for confirming/infirming public health threats and emergencies. It is necessary to mention that the equipment from the SPHSS laboratories is obsolete and it needs to be replaced by modern equipment so as to improve the capacities for identifying and confirming the threats.

41. Currently, there is no inter-sectorial National Committee for Biosecurity and Biosafety so as to develop the legal framework in the area of biosafety and biosecurity for creating a National Register of Laboratories identifying, assessing, monitoring, and minimizing the risks of biologic origin, including preventing bioterrorism. The National Commission for Biological security, established through the Law nr. 755-XV from 21st December 2001 on biological security, executes only authorization and control duties of activities involved in process of obtaining, testing, producing, using and marketing of genetically modified microorganisms, plants and

animals through modern biotechnology, according to the Regulation of organization and functioning of the nominated Commission approved by the Government Decision nr. 603 from 20th May 2003.

Section 4. Health protection (*environmental health, occupational health, food safety, nutrition, and other areas*)

42. Health protection represents an inter-disciplinary and cross-cutting area that needs the involvement and cooperation of all authorities and stakeholders at the national and local levels.

43. Risk surveillance is carried out mainly by the SPHSS; the public health laboratories (physical, chemical, microbiological, parasitological, and radiological) investigate environmental factors, including occupational factors, foodstuff, and other products. The microbiological and virological labs investigate the biological samples collected from patients diagnosed with infectious diseases, as well as for reasons of epidemiological surveillance. All the labs have internal quality control programs, and the test procedures are described in the quality manuals. In spite of the fact that there is a well-developed network of risk surveillance, it is necessary to mention about their reduced capacities, and namely insufficient equipment and limited training of the lab personnel, which impedes the implementation of reforms and new testing methods.

44. The process of legislation development and harmonization is led by the responsible authority in cooperation with other interested authorities. In case of water supply and sewerage – the Ministry of Health is responsible for developing national legislation in relation to drinking water, and in collaboration with the Ministry of Environment – for developing the legislation on sewerage and discharge of waste and residues. The Ministry of Environment in collaboration with the Ministry of Health is responsible for developing the legislation on air quality and environment protection. The Ministry of Labor, Social Protection and Family is the main authority for developing the legislation in the area of occupational health and safety, while the Ministry of Health is mainly responsible for monitoring and evaluating the occupational health. The support legislative framework in the area of health protection is incomplete, but it is being developed and aligned to the legislation of European Union. Big reserves exist in the area of the legislative and normative frameworks' enforcement.

45. The activity of the food safety system, as well as other activities in the area of health protection are cross-cutting, including responsibilities for inspection, monitoring, and enforcement, which are divided between the newly-established National Food Safety Agency (NFSA), under the Government of the Republic of Moldova, the SPHSS under the Ministry of Health and the Consumer Protection Agency (CPA) under the Ministry of Economy. The existing legislation and a part of the related regulations are in line with the EU legislation.

46. The surveillance over production, labeling, trade, and use of cosmetics, toys, non-food products is carried out in line with the national legislation, which is not yet harmonized with the EU legislation. In this context, a draft Government Decision transposing the EU legislation on cosmetics into the national legislation was developed and is to be approved.

47. The risk assessment for biocide products is carried out by the Cross-sectorial Commission of Experts for registration of biocide products under the Ministry of Health; and the biocide products are produced,

labeled, and placed on the market in compliance with the regulation in force, which is harmonized with the EU Directives.

48. Nutrition represents a relatively new area for the Republic of Moldova. Currently, under the SPHSS reorganization process, this area was attributed to the responsibility of the specialists from this sector. Currently, a National Plan of Actions on Foodstuff and Nutrition is developed and a nutritional surveillance system is being implemented. Two programs for fortification of food products have been approved and are being implemented (the salt with iodine and flour with iron and folic acid).

49. The quality and safety of healthcare services are regulated by standards, protocols, and guidelines approved by the Ministry of Health. The healthcare facilities providing healthcare services are accredited by the National Council for Evaluation and Accreditation in Health (NCEAH), under the Ministry of Health. The activity of private medical institutions is subject to licensing. The accreditation of the public medical-sanitary institutions did not become, de facto, an essential criterion for their operation. A weak system of internal control for nosocomial infections within the medical institutions and the lack of external control for the quality of the provided medical services are among the problems which were not yet settled.

Section 5. Health promotion

50. Health promotion is the process which empowers the persons to increase control over their health status and health determinants, thus improving their health. This aspect is an important element of public health, which tackles communicable and non-communicable diseases.

51. The activities related to health promotion, advocacy, communication, and community mobility for health are just at an incipient development stage in the Republic of Moldova. Starting in 2007, higher attention was granted for health promotion activities – this was the year when the National Program for Promotion of Healthy Lifestyle was adopted for 2007-2015. In spite of the multiple actions included in the Program, many of them are implemented only partially. To implement this Program, the Ministry of Health collaborates with the Ministry of Education in promotion of healthy nutrition, physical activity and education for health in schools, as well as with the State Labor Inspectorate in the area of health promotion at work and decrease of occupational diseases and work accidents, but this collaboration is insufficient. The level of knowledge about reproductive health is rather low: only 38.2% of young persons aged 15-24 years old prove to have the corresponding knowledge about HIV/AIDS. About 15% of abortions were registered among teenagers aged 15-19 years old, and this is partly induced by the limited access of youth to education for sexual and reproductive health.

52. A number of projects was developed and implemented at the community level to tackle some health promotion strategies, but they are not implemented on regular basis and at a national scale.

53. A number of health promotion activities are supported financially by the National Health Insurance Company (NHIC), but these activities are not consistent and don't have a well determined aim; these activities are not planned and assessed based on CAP evaluation surveys.

54.Primary Healthcare (PH) plays an important role in the implementation of public health strategies for disease prevention. Nevertheless, currently these activities continue to be focused on treatment, except for vaccination, counseling and monitoring pregnant women, as well as monitoring babies' and children's health. PH doctors, all by themselves, do not have enough knowledge and capacities to consult and help people to practice healthier lifestyle, to identify the persons at risk and to prevent these risks.

Section 6. Prevention of diseases, including early identification of diseases

55.Prevention of diseases is focused on communicable diseases, as well as on non-communicable diseases, implying specific person-centered actions. The disease prevention services should include primary prevention through population vaccination; application of vaccination or post-exposure prophylaxis in case of the persons exposed to communicable diseases.

56.The Ministry of Health coordinates the National Immunization Program (NIP). The ministry should ensure the integration of the NIP into the NHP and other sector policy initiatives, including also those that involve the health insurance system and the minimum package of medical services. The NIP is coordinated by the Ministry of Health along the participation of the Coordination Council for NIP financing and development directions, the NPHC, the State University of Medicine "Nicolae Testemitanu", the NHIC, and other medical institutions.

57.The NPHC is authorized by the Ministry of Health to perform the general management of the NIP and to ensure the link with other departments from the health sector.

58.The SPHSS ensures the development of immunization programs; planning; monitoring and evaluation of their implementation; management of the immunization system (needs and delivery timetable, reception, storing, distributing, monitoring, and maintaining the stocks of vaccines and consumables, etc.). The Ministry of Health together with other ministries, including Ministry of Education develops guidelines and procedures for ensuring the coverage of the population with immunization. The primary prevention of the communicable diseases and some non-communicable diseases through vaccination of children, adults and elderly people in line with the immunization programs, and post-exposure prophylaxis of the persons exposed to risks are implemented by the primary healthcare services. The NIP for 2011–2015 provides for immunization against 12 infections: viral hepatitis B; TB; diphtheria; tetanus; pertussis; poliomyelitis; measles, mumps and rubella; *Haemophilus influenzae* of type b; and infections with rotavirus and pneumococcus. A national deposit of vaccines, established within the NPHC and certified by WHO, provides optimal conditions for storage of vaccines. Vaccination is provided to citizens free of charge and the Republic of Moldova maintains high immunization rates, in spite of the anti-vaccination campaigns. The NPHC works together with the territorial public health centers to manage the NIP implementation, as well as with the PH services and other relevant agencies from health sector. The services of PH, the PH centers, and family doctors are responsible for providing immunization services to the population.

59.Prevention and control of diseases related to lifestyle – focused on tobacco and alcohol control activities – represent a priority for the country. The national programs for tobacco and alcohol control were developed and approved by the Government in the first quarter of 2012. In compliance with these programs,

some specific services will be created and implemented, such as counseling services for giving up smoking and counseling for the persons who use alcohol in harmful volumes. These services will be probably funded by the Compulsory Health Insurance Funds (CHIF) from the NHIC, which will conclude contracts with the service providers. But there is no system of easily available services for drug users.

60. The prevention of congenital malformations and anemia is carried out through standard monitoring of pregnant women during the pregnancy. Since 2001, during pregnancy and six months after birth, women receive folic acid and iron supplements for free (the costs are covered by the NHIC). In spite of the free-of-charge distribution of these supplements, the undertaken surveys reveal high levels of anemia among pregnant women (43.0%) and women of reproductive age (27.3%). To reduce these deficiencies, the Government has approved the first National Program for reducing the diseases determined by iron and folic acid deficiency for 2012–2017. According to this Program, all the flour used in the food industry should be fortified with iron and folic acid.

61. Pregnant women are subject to prenatal screening for Down syndrome and other chromosome disorders. In maternity hospitals, the new-born children are subject to screening for phenylketonuria and the inborn loss of hearing. The prevention of hemorrhagic diseases by prescribing the vitamin K to the new-born children is not included. To inform better the pregnant women, besides the standard consultations, all the women receive a perinatal medical agenda during the first antenatal assistance visit. This agenda includes the standards for examination, consultation, antenatal intervention, and educational messages for mothers. The pregnant women also receive the “Guide of the mother-to-be”, which provides information regarding the development of the child from conception till birth, along some recommendations on how to take care of the child, how to feed the child, and other information. Nevertheless, the level of provision of educational materials about pregnancy and childcare is inadequate.

62. Reproductive health should be included on priority basis in the National Public Health Strategy. Even though it follows an obvious continuous decreasing trend, the abortion rate is still very high and it is still a method for fertility regulation. The prevalence of contraceptives is at a much lower level as compared to the European countries, and this is explained by the lack of information regarding the modern methods, the high cost or non-availability of such methods for the vulnerable groups. It is important to clarify the status, role, as well as the efficiency of services provided by the reproductive health offices in the context of primary healthcare and family medicine consolidation. The youth friendly services need to be extended at the national level.

63. The programs for cancer screening (cervical and breast cancer) among the population are carried out during some special annual campaigns supported by the NHIC. However, they cover only a part of the population. The screening for cervical cancer was introduced as part of the regular gynecological practice; but it is not carried out on systematic basis and there are no quality control and quality assurance standards. Currently, there are no screening programs for colorectal and prostate cancer.

64. Prevention, surveillance and control of nosocomial diseases is a very large area and the SPHSS duties include the development of the normative framework for creating and maintaining a safe and inoffensive environment for the patients who are provided with medical assistance, including under the aspect of injections’ safety, management of waste resulting from medical activity, etc. The actions for preventing the transmission of nosocomial infections represent a responsibility of the medical-sanitary institutions’ heads, and

the monitoring and evaluation aspects are attributed to the SPHSS for the purpose of assessing the actions undertaken to prevent the transmission of communicable diseases to patients and health workers.

65. Prevention and reduction of development of drug-resistant microbes may be achieved by approving strategies for prudent use of antibiotics in the treatment of infectious diseases in humans and animals, as well as by regulating the use of anti-microbial preparations in food products as preservatives and in fodder used for feeding animals. The development of clinical protocols with necessary adjustments, as well as limitation of free trade of preparations, monitoring of resistant strains, regulations and norms for the admissible concentration of antibiotics in food products – are cross-sectorial activities and will be carried out by the Ministry of Health (SPHSS, Medicine Agency) and Ministry of Agriculture and Food Industry.

66. Prevention of infectious diseases outbreaks shall be carried out by maintaining the capacities for rapid detection of the risk aggravating the epidemiologic situation, referring to the appearance of one single case or a cluster of cases in the same place and the same period, epidemics or pandemics. An efficient surveillance system allows the early detection of outbreaks, assessing the damages and the needs for preventing the spread of the event and controlling the outbreaks by ensuring an adequate preparedness level (e.g. stocks of medicines, standard protocols for treatment and protocols for personnel training) and rapid reaction (e.g. confirmation, investigation, including through lab method and application of control measures).

Section 7. Staffing the public health sector with competent and sufficient human resources

67. In the Republic of Moldova, the mid- and long-term policies related to health human resources cover certain components of human resources' management and are reflected in the Program for Development of Medical and Pharmaceutical Education in the Republic of Moldova for 2011-2020, the Program for Strategic Development of the Ministry of Health for 2012-2014, the Framework Concept for Human Resources' Development in the Health System, etc.

68. The improvement of the population health status depends to a large extent, on the quality and training of employees working in public health sector, which depend, at their turn, on the relevance and quality of education and training. Over the last 10 years, the Republic of Moldova encountered problems with ensuring competent personnel for the public health sector. Thus, an insufficient number of medical professionals is registered in general, and certain specialists in some specific areas, in particular. The number of doctors and nurses working in public medical institutions has decreased by 38% and respectively 50% since 1994 until 2010, inducing a real challenge for the health system. There are too many medical professionals in certain disciplines and in urban area, while at the same time, there are too few epidemiologists, public health professionals, laboratory technicians, and family doctors in rural communities. Although higher salaries were offered to health workers in rural area, unfortunately the problem of understaffing still persists.

69. The public health specialists usually obtain official qualifications after graduating the Public Health Faculty of the State University of Medicine and Pharmacy „Nicolae Testemitanu” and after finishing the

residency in public health area. The postgraduate training courses in public health area, including in the area of hygiene, epidemiology, and health management are available for the health professionals at the same university. Since 2011, the NPHC provides short-term training courses for some specific subjects, in line with the continuous training program.

70. The School of Management in Public Health was established in 2003 within the State University of Medicine and Pharmacy “Nicolae Testemitanu” and it is an active member of the Association of Public Health Schools in the European Region. Until now, over 200 postgraduate students (master degree) in public health have graduated from this School. Before 2007, only health professionals were trained within this School, while since 2007 the School provides training courses also for non-medical professionals from health area.

71. The main barriers for improving the training of employees in public health area refer to the inadequate quality and quantity of training centers and lack of trainers and up-to-date programs. It is necessary also to have evidence-based training materials, which would facilitate the access to modern technologies and medical protocols focused on public health.

72. The multiple reforms undertaken within the current SPHSS, as well as the repeated integral revision of the legislation in public health area had an advanced impact on the dynamics of the Service's development. After 1991, the State Sanitary-Epidemiologic Service was transformed into the Hygiene and Epidemiology Service, which subsequently was transformed into Preventive Medicine Service, and finally in 2009 it became the State Public Health Surveillance Service, which is similar to the public health structures from the European countries and in line with the WHO recommendations. All the perpetuated reforms of the SPHSS have resulted in essential cuts of Service personnel. During the respective period of time, the prophylaxis of diseases and health promotion benefited from an extremely reduced financing.

Section 8. Scientific research in public health

73. The active use of scientific evidence in developing policies and increasing the capacity of the public sector to collect, analyze, and disseminate information on health are important elements of activities related to management, fulfillment, and implementation in public health area. Over the last years, the national authorities acknowledged the research importance for the rapid development of public health.

74. The double subordination of the research institutions – to the Academy of Science and the Ministry of Health, reduces the autonomy of the institutions, hence impeding the continuous development of the strategic research capacities and discouraging their competitiveness.

75. The majority of national research projects are funded by the Moldova Academy of Science (MAS), according to the Code on Science and Innovation of the Republic of Moldova No. 259, dated July 15, 2004, as a result of competition results, but it is insufficient and has been decreasing over the last 5 years. Thus, the share of public funds going to research from GDP accounted in 2004 for 0.22%, increasing by 2008 – 0.7% and afterwards decreasing by 2011 – 0.4%.

III. VISION, AIM, GENERAL AND SPECIFIC OBJECTIVES

76. Vision: sustainable health and wellbeing through enhanced public health capacities and services.

77. Aim: to improve population health and reduce inequities in health by enhancing the State Public Health Surveillance Service to carry out essential public health operations and provide accessible and high quality public health services at the individual, community, and population levels.

78. General objective: to develop, implement, and evaluate the actions for enhancing public health capacities and services.

79. Specific objectives:

1. to make more efficient and to consolidate the population health surveillance systems so as to identify the health problems and to supply relevant, accurate, and timely information for decisions and actions in public health area;
2. to strengthen the national system for prevention, preparedness and response in public health emergencies through integrated approach to threats;
3. to ensure health protection by making more efficient the control over behavioral and environmental risk factors;
4. to adopt some healthy behaviors by the population of the Republic of Moldova through implementing efficient and coordinated measures for health promotion by the different sectors at the national and local levels;
5. to reduce the burden of communicable and non-communicable diseases by reducing the risk factors and ensuring equitable access of population to primary, secondary, and tertiary prevention services;
6. to improve the cross-sector collaboration mechanisms with clear definition of responsibilities among the health sector and other sectors in implementing the essential public health operations;
7. to ensure with sufficient and competent human resources in public health area by strengthening the training system for undertaking the essential public health operations;
8. to adjust the organizational structure and to improve the financing of the public health surveillance service so as to carry out the essential operations and services in public health area in collaboration with other sectors;
9. to consolidate research in the area of public health so as to ensure evidence-based reasoning for policies.



IV. AREAS OF PRIORITY INTERVENTION

Section 1. Population health surveillance

80. Problems identified in population health surveillance area:

1. the following was identified in the collection, storage, and processing of statistical data carried out at the national level by several institutions – NBS, NCHM, NPHC, and NHIC: doubling of some data collection, inefficient and limited communication, collaboration, and coordination, as well as reduced possibilities to integrate the institutional information systems into an integrated information system (due the incompatibility of some systems);
2. data on health determinants and risk factors, especially in case of non-communicable diseases, are not collected systematically, and the existing data are incomplete and reflect just a part of problems. The data from surveillance and control systems for communicable diseases, including for sexually-transmitted diseases, TB and HIV/AIDS are managed inadequately due to fragmented reporting and existence of parallel data reporting systems;
3. the collected data do not contain all of the necessary characteristics and are not disintegrated to identify inequities in health and to develop interventions to diminish them;
4. there is no complex vision regarding the systematized conduct of population-based surveys, frequency of such surveys, who should carry them out, and the availability of financial means to cover the costs;
5. there are no national registers for priority non-communicable diseases (cardiovascular diseases, diabetes, mental health, etc.), while the existing registers operate rather unsatisfactorily (National Register for Cancer);
6. there are no sufficient skills at the administrative-territorial unit level to carry out population health status assessments and to process and manage the data, as well as to develop necessary measures;
7. the definition of indicators and processing of data are not standardized; the system for verifying data quality is missing, and this fact does not ensure comparability of data at the national and international levels.

81. Specific objective: to make more efficient and to consolidate the population health surveillance systems so as to identify the health problems and to supply relevant, accurate, and timely information for decisions and actions in public health area.

82. The following tasks are set forth to fulfill the specific objective:

1. to enhance and increase the efficiency of the surveillance system for diseases and population health status;
2. to monitor and periodically evaluate population health status, communicating the obtained results to interested authorities and population.

83. Strategic interventions to be carried out:

1. to revise and standardize the health indicators currently used, and to disaggregate them according to social-economic criteria, sex, level of education, area of residence, region, and ethnic groups;
2. to harmonize the content and methodology for collecting, processing, and communicating data on health status and health determinants in compliance with the international rules;
3. to develop and enforce the data quality management system in the area of public health;
4. to develop and implement the integrated electronic system for surveillance of non-communicable diseases and risk factors;
5. to adjust and integrate the systems of epidemiological surveillance and communicable diseases' control including TB, HIV/AIDS, and sexually transmitted diseases;
6. to develop and enforce agreements with relevant national structures on data access and information sharing, and to establish the ways for data collection and data quality monitoring;
7. to build capacities at the local and national levels for assessing population health status and improving analysis reports on systematic basis;
8. to participate in international studies on surveillance of health status and risk factors;
9. to ensure communication of the obtained results of population health and risk factors' surveillance to interested authorities, population, and international organizations.

Section 2. Public health emergencies

84. Problems identified in public health emergencies:

1. the legal framework in the area of preparedness and response to public health emergencies is not fully adjusted to the provisions of EU documents. The missing standard operation regulations/procedures for intervention in case of hazards and emergencies, as well as the insufficient funding for prevention, preparedness and management of public health emergency plans represent one of the existing challenges;
2. the dispersed capacities to identify, assess, and manage public health risks and emergencies, as well as the insufficient cross-sector collaboration at the local and national levels maintain the danger for public health security and safety. The preparedness and response in cases of public health emergencies need a generic approach;
3. there are deficiencies in equipping and training the intervention teams in line with the international standards for managing public health exceptional situations (planning, preparedness, assessment of needs, vulnerability, risks, management, communication, post-intervention evaluation, etc.), which should be redressed;
4. there are limited, as a number and a training level, a human resources in primary healthcare and public health system, so as to able to assess the risks, to carry out feasibility and cost-efficiency studies, as well as to report data to the surveillance system.

85. Specific objective: to strengthen the national system for prevention, preparedness and response in public health emergencies through integrated approach to threats.

86. The following tasks are set forth to fulfill the specific objective:

1. strengthening the national capacities for early detection and rapid response to public health threats and emergencies based on risk monitoring indicators at local, intermediary (district/municipality) and national levels;
2. minimizing the negative impact on population health, determined by public health risks and emergencies caused by natural, technogenic, biological, radiological, and social factors.

87. Strategic interventions to be carried out:

1. to improve the legal framework through integrated and generic approach in the area of preventing,

preparing, and responding to public health emergencies caused by natural, technogenic, biological, and social factors, and bioterrorism;

2. to develop and implement the framework related to the National System for health sector's preparedness and response to public health emergencies;
3. to create the framework and to ensure the functioning of the Operational Center for managing public health emergencies, as part of international networks, including for the implementation of the International Sanitary Regulation 2005, INFOSAN, ChemiNet, GladNet, etc.;
4. to establish mechanisms for sustainable financing for the implementation of the plans related to prevention, preparedness, and response to public health emergencies;
5. to continuously enforce the system of early warning and rapid response for prevention and control of communicable diseases and public health events;
6. to involve central and local public authorities, within the limits of their duties and means, in activities related to planning, preparedness, and management of public health risks and emergencies;
7. to increase the efficiency of the response activities related to planning, preparedness, evaluation, management, and communication of risks, post-intervention evaluation;
8. to strengthen national capacities in biosecurity and biosafety areas;
9. to enhance the SPHSS lab network for rapid identification of biological, chemical, and radiologic agents;
10. to continuously develop national capacities for early detection and rapid response in case of public health events of chemical origin.

Section 3. Health protection

88. Problems identified in health protection area:

1. incomplete legal framework;
2. limited capacities at the local and national levels for developing and enforcing the regulatory framework, and for monitoring the implementation in the health protection area;
3. procedures for inspecting, monitoring and enforcing the legislation are obsolete; they are not based on risk assessment principles;
4. collaboration between the responsible authorities is insufficient, each of them acting in line with their own monitoring and evaluation plans;
5. deficient staffing with human resources, limited technical endowment of the SPHSS, and insufficient training of the personnel represent impediments for implementing new methods;

6. the system of surveillance and control in occupational health is going through a reorganization process. Currently, the responsibilities of the different authorities, services, employers, and employees are not clearly defined in this area. The segment of occupational health has not been improved within the SPHSS, there are no national programs in occupational health area, and other;
7. there are no policies in the area of nutrition and food provision for different groups of population. The implementation of the provisions set forth in food safety documents and food products' labeling is incomplete;
8. the financial resources allocated for identifying safe drinking water sources in rural localities are insufficient. No systems are implemented for making drinkable the water which does not comply with standards, due to the high costs and missing trained personnel;
9. the regulatory framework on quality of atmospheric air is not aligned to the EU regulations and WHO recommendations; deficiencies are registered in assessing the atmospheric air. There are no capacities to determine the content of some priority pollutants (PM 10, PM 2.5 and other);
10. there are no adaptation strategies and response plans at national and local levels, including those of the health system, to the extreme phenomena of climate change;
11. the technical capacities for assessing the risks conditioned by the use of cosmetics, toys, and the risks from chemical safety area are limited;
12. the quality and safety of healthcare services provided to patients are insufficiently ensured. Not all medical institutions have an efficient system for infections' control, and the existing ones are poorly implemented;
13. the system for waste management, including medical waste, is deficient and thus affects population living conditions and health.

89. Specific objective: to ensure health protection by making more efficient the control over behavioral and environmental risk factors.

90. The following task are set forth to fulfill the specific objective:

1. to improve the legal and regulatory framework in health protection area by harmonizing it with the international regulations;
2. to enhance the SPHSS capacities to assess, manage, and communicate the information about the public health risks and to establish the mechanism for collaboration with other services in this area;
3. to strengthen the capacities for identifying and managing the public health risks of other authorities with duties in the area of health protection (environmental health, occupational health, food safety).

91.Strategic interventions to be carried out:

1. to harmonize and adjust the legal framework in health protection area with the international regulations;
2. to develop mechanisms for enforcing legislative and normative acts, monitoring their implementation, and evaluating their impact;
3. to assess the implementation results of programs, action plans in environmental health area and to update them based on the commitments assumed in Parma in 2010;
4. to develop the policy framework on adaptation to extreme phenomena of climate changes; to build institutional capacities and to strengthen the occupational health system;
5. to develop policies on food provision and nutrition of different population groups, focused on reducing the content of salt, fat and trans fat acids in the processed foodstuff;
6. to modernize the SPHSS labs by increasing their capacities to identify and monitor risks in health protection area;
7. to strengthen the capacities for assessing the risks conditioned by the use of cosmetics, toys, and risks from chemical safety area;
8. to strengthen the measures for patients' safety and infections' control system.

Section 4. Health promotion

92.Problems identified in health promotion area:

1. the National Program for Promotion of Healthy Lifestyle is not covered financially; the resources allocated by the State, as well as the compulsory health insurance funds are not enough to cover the costs for developing and implementing health promotion activities;
2. the current National Program for Health Promotion is obsolete, it needs to be revised and updated, and some activities included in health programs are too general;
3. the programs and information materials on health promotion and health education are developed sporadically, insufficiently and mainly within national or local projects funded by donors;
4. there are no surveys related to risk perception at workplaces so as to assess the existing knowledge and practices, to plan promotion and communication actions on assessment basis;
5. there are no information and education materials on health issues in the area of occupational health;
6. communication for behavioral change, information and protection of reproductive health are not priorities in the activities carried out by public medical-sanitary institutions and schools;

7. life skills based education was not encompassed in the compulsory school program;
8. the involvement of the primary, hospital, and specialized health assistance in health promotion activities is insufficient;
9. the involvement of other sectors, including that of the local public authorities and mass-media, in health promotion and support is insufficient at the local level;
10. the financial, human, and material capacities for planning and implementing health promotion activities are limited.
11. there is no mechanism for coordination and financing of national and local interventions on health promotion.

93. Specific objective: to adopt some healthy behaviors among the population of the Republic of Moldova by implementing efficient and coordinated measures for health promotion by the different sectors at the national and local levels.

94. The following tasks are set forth to fulfill the specific objective:

1. to consolidate the health sector's infrastructure and capacities in health promotion area;
2. to increase the potential and involvement of other sectors than health sector in activities for health promotion and improvement of inter-sector collaboration at the national and territorial/local levels;
3. to create health environments by establishing partnerships with civil society and empowering communities to plan and implement efficient health promotions activities.

95. Strategic interventions to be carried out:

1. to enhance the legislative and regulatory frameworks providing support for facilitating healthy choices, adhering to healthy lifestyles and creating healthy environments, by respecting criteria of equity;
2. to ensure sustainable and continuous financing for health promotion programs at the territorial and national levels;
3. to consolidate the SPHSS for ensuring planning, development, implementation, and assessment of health promotion activities at the population level;
4. to enhance the primary health care functions in providing health promotion services, along the curative ones, at the individual level;

5. to organize health promotion and disease prevention activities at the community level by establishing a special unit in the primary healthcare units;
6. to enhance the capacities of other sectors and local public authorities to develop, implement, and evaluate the health promotion activities and healthy environments;
7. to improve inter-sector cooperation and collaboration of the Ministry of Health with central and local public authorities in health promotion area;
8. to create funds for supporting the implementation of health promotion initiatives at the local and national levels, to manage efficiently the available funds;
9. to create sustainable partnerships with NGOs and civil society for carrying out health promotion activities.

Section 5. Prevention and early detection of diseases

96. Problems identified in prevention and early detection of diseases area:

1. the policy documents on priority non-communicable diseases (e.g. diabetes) do not fully elucidate the prevention measures, especially the primary prevention ones. The existing national programs are mainly focused on disease management, while the measures focused on prevention, identification, and control of risk factors are insufficient or missing;
2. involvement of other sectors in control of risk factors is limited and provided only under several national programs (e.g. reduction of alcohol and tobacco use, diseases determined by deficiency of iodine, iron, and folic acid);
3. primary prevention of some communicable and non-communicable diseases through vaccination needs sufficient and sustainable financing, assurance of immunization system's maintenance and increase of vaccination coverage rate (in case of vaccine-preventable diseases);
4. the measures for early detection of pre-morbid conditions and some diseases, mainly non-communicable ones, in incipient stages are implemented partially, only for some groups of population and territories. No continuity of such measures is ensured in the health sector;
5. service quality discrepancies, unfair access to primary prevention services, treatment and rehabilitation services are registered in different territories;
6. there is a low level of knowledge and practices among health workers in relation to prevention services' provision: identification and reduction of risk factors, early detection of diseases, disease case management, and continuous monitoring for preventing diseases' progress and appearance of sequelae, etc.;

7. there are no evaluation mechanisms for the efficiency of disease prevention measures undertaken by the primary, hospital, and specialized healthcare for improving population health at locality, rayon/municipality, and country levels;
8. human resources and specialists' skills are limited in the SPHSS to develop, implement, monitor and evaluate activities related to disease (especially non-communicable diseases) prevention and risk factors' reduction;
9. the financial resources allocated for the implementation of prevention measures at the national and local levels are inadequate.

97. Specific objective: to reduce the burden of communicable and non-communicable diseases by reducing the risk factors and ensuring fair access of population to primary, secondary, and tertiary prevention services.

98. The following tasks are set forth to fulfill the specific objective:

1. to decrease the risk factors at the individual, community, and national levels through integrated activities of health sector together with other sectors and local public authorities;
2. to prevent and control vaccine-preventable diseases by covering the target groups with vaccines in line with the provisions of the national immunization programs and to decrease communicable diseases' morbidity by 2% by 2020 via measures of primary and secondary prevention;
3. to ensure fair access to quality services of disease prevention provided by primary, hospital, and specialized healthcare and public health surveillance service.

99. Strategic interventions to be carried out:

1. to improve the legal framework in the area of communicable and non-communicable diseases' control and prevention, including control of risk factors, by ensuring a generic and integrated approach to intervention measures and reducing inequities;
2. to provide sustainable and continuous financing for communicable and non-communicable diseases' control and prevention programs at the national and territorial levels;
3. to ensure cross-sector cooperation and collaboration within the mandate limits of the Ministry of Health, Ministry of Agriculture and Food Industry, Ministry of Internal Affairs, Ministry of Environment, Ministry of Education, Ministry of Transport and Road Infrastructure and local public authorities in the area

of communicable and non-communicable diseases' prevention, including risk factors' decrease;

4. to strengthen the capacities of public health service, local public authorities together with other relevant sectors in such areas as population health assessment, prioritization of problems, planning of complex intervention measures, monitoring of implementation, and evaluation of interventions' efficiency;

5. to enhance the knowledge and practices of health workers with secondary and higher education about early identification of diseases, associated risk factors, and their control;

6. to detect early the risk factors and pre-morbid situations, or incipient disease conditions by maintaining and strengthening national systems for surveillance and control of communicable and non-communicable diseases;

7. to organize periodically population-based surveys related to the most important problems of public health;

8. to implement fair and safe state-guaranteed vaccination programs for different age groups;

9. to create and maintain safe and inoffensive environment for patients by strengthening surveillance and control of nosocomial diseases, including by decreasing the resistance of microorganisms to an antimicrobial medicine;

10. to design and implement the concept of counseling for risks' reduction and diseases' prevention;

11. to improve youth access to education and counseling in sexual and reproductive health through reproductive health offices and youth friendly service centers;

12. to set partnerships with civil society organizations and to enhance their capacities to mobilize and empower community networks to carry out the health protection and promotion of healthy lifestyle activities.

Section 6. Assurance of population health governance

100. Problems identified in assurance of population health governance area:

1. reduced efficiency to promote "health in all policies" principle;

2. inequities are registered in population access to primary health care services because of the insufficient number of doctors in rural area and a more reduced wellbeing level of the population; the access to health services in case of vulnerable and disadvantaged groups (such as elderly people, disabled people, unemployed persons, etc.) is limited;

3. the medical institutions don't have well-defined and standardized quality indicators for monitoring, evaluating, and comparing the quality of provided medical services, and the knowledge of health managers about the quality management in health care facilities is insufficient;

4. internal and external audits of the medical and public health services are carried out sporadically, mainly at the territorial level, with limited complexity and unilateral recommendations.

101. Specific objective: to improve the cross-sector collaboration mechanisms with clear definition of responsibilities among the health sector and other sectors in implementing the essential public health operations.

102. The following tasks are set forth to fulfill the specific objective:

1. to strengthen collaboration between the health sector and other sectors so as to reduce the risk factors and improve population health status, applying the “health in all policies” principle;
2. to improve the quality of medical and public health services, to increase the efficiency of interventions and to reduce health inequities;
3. to implement e-governance mechanisms in public health services.

103. Strategic interventions to be carried out:

1. to develop and implement a mechanism for collaboration with other sectors (education, transportation, labor and social protection, constructions, agriculture, etc.) and civil society for developing, implementing, monitoring, and evaluating public health programs;
2. to increase the knowledge and responsibilities of local public authorities, involving them in organization of public health activities at the local level through: evaluation of population health status and development of health profiles; planning, implementation and evaluation of activities focused on reducing risk factors and improving population health status;
3. to develop partnerships at the national and local levels between different sectors and civil society so as to implement activities which aim to reduce risk factors and to improve population health status;
4. to assess periodically the implementation of policies and programs with impact on population health, including their funding and to publish and disseminate the results;
5. to develop standardized indicators for assessing the quality of public health services and to adopt the quality standards and common definitions established at the European level (WHO and EUROSTAT);
6. to evaluate periodically the activities of medical and public health institutions based on standardized indicators and to disseminate the evaluation results;
7. to ensure adequate and sustainable financing for public health programs;
8. to develop mechanisms for distribution and control of financial means, envisaged for prevention of diseases and promotion of health by the NHIC;
9. to implement e-governance mechanisms and information systems in public health area;
10. to ensure information exchange with other national and international systems of public health services.

Section 7. Staffing the public health sector with competent and sufficient human resources

104. Problems identified in staffing with human resources:

1. the process of personnel professional training for public health system is not fully in line with the society exigencies and international standards; the professional training roadmap is not defined;
2. the level of professional training does not correspond to the capacities and skills demanded by the public health system;
3. the classifier of specialties tackles the place of the public health area erroneously;
4. resistance is registered when trying to involve non-medical personnel and employees with secondary education in the public health system;
5. there is no methodology for evaluating the needs and planning human resources for the public health system for mid- and long-term, depending on the population health status, economic trends, demographic changes and other;
6. a misbalance is registered between the staffing with human resources from geographic and areas of residence point of view; there is a deficit in number of young specialists;
7. there are no clear and attractive opportunities for specialists from public health area; the system for personnel motivation is deficient and it does not include decent incomes, adequate work conditions and possibilities for professional promotion based on some objective criteria.

105. Strategic objective: to staff the public health area with sufficient and competent human resources, by strengthening the training system for undertaking the essential public health operations.

106. The following tasks are set forth to fulfill this specific objective:

1. to staff the public health system structures with sufficient numbers of qualified personnel and to enhance the legislative framework; to estimate the needs and to plan human resources for carrying out essential public health operations;
2. to modernize the process and stages of professional training, adjusting the curricular components of the pre-university, undergraduate, postgraduate, and continuous training of the personnel in line with the essential competences for the public health set by the Association of Schools of Public Health in the European Region (ASPHER);

3. to staff the public health system with human resources in a sufficient and balanced way by developing and implementing innovating programs for recruiting, maintaining, and developing the personnel.

107. Strategic interventions to be carried out:

1. to develop the legal framework and the concept of specialists' training in public health area;
2. to estimate the needs and to plan human resources for short, mid- and long-term, in line with the possible changes (economic trends, demographic changes, etc.);
3. to revise the process of professional training from the following points of view: stages, duration, continuity, and synchronization of training programs;
4. to provide training and career opportunities in public health area for the specialists from non-medical areas;
5. to enhance the teaching capacities in the public health area from number and training level perspectives;
6. to evaluate and adjust the curricular components of the pre-university, undergraduate, postgraduate, and continuous training programs in line with the essential competences (surveillance, public health emergencies, promotion and protection of health, and disease prevention) for public health established by ASPHER and the national needs in compliance with the reforms of the public health system;
7. to update the standards for assessing the training and qualification level of the medical and non-medical personnel in the public health area; to evaluate periodically the level of training and qualification of the medical and non-medical personnel from the public health area and to train them in line with the identified needs;
8. to develop and implement efficient mechanisms for recruiting medical and non-medical employees in the public health area and from other systems providing health services;
9. to develop and implement motivation/incentive programs for the employees from the public health system and from other systems providing health services.

Section 8. Organizational structures and financing of the State Public Health Surveillance Service

108. Problems identified in State Public Health Surveillance Service organization and financing:

1. the new trends in public health are not fully understood by all the national decision-makers; there is

no single approach to public health in the Republic of Moldova and the roles and responsibilities of all stakeholders are not clearly defined;

2. the frequent policy changes and the lack from the very start of some clear strategies and rigorously defined objectives had a negative impact on the health system, in general and on the public health system, in particular;

3. there is no clearly defined legal framework which would delimit the functions, duties, and structures related to disease prevention and health promotion by primary, hospital, and public health services at the locality and district levels;

4. the public health system reforms (4 in total) which have involved actively all the partners for health protection, health promotion and disease prevention did not result in increasing the role of local public authorities, professional associations, financing institutions, and communities; and the delay of the administrative-territorial regionalization does not allow the regionalization of the State Public Health Surveillance Service structure, including the fortification of the lab control system by improving the finding of the public health system, optimization of expenditures, increase of quality and access to adequate public health services;

5. there is no efficient collaboration and partnership with the National Health Insurance Company in public health issues, including for the implementation of the National Program for Promotion of Healthy Lifestyle and other national programs.

109. Specific objective: to upgrade the organizational structure and to improve the financing of the State Public Health Surveillance Service so as to carry out the essential operations and services in public health area in collaboration with other sectors.

110. The following tasks are set forth to fulfill the specific objective:

1. to consolidate the State Public Health Surveillance Service and other health services at the national and territorial levels for ensuring the fulfillment of essential public health operations and services;

2. to ensure sustainable financing for the State Public Health Surveillance Service for fulfilling essential public health functions and activities.

111. Strategic interventions to be carried out:

1. to gradually reorganize the State Public Health Surveillance Service structure, piloting the reform proposals, and to carry out a situational analysis;

2. to monitor the activities and assess the efficiency and effectiveness of the pilot public health center and performance labs;

3. to strengthen the performance labs of the public health centers, equipping them with modern devices and technique;
4. to develop performance indicators for the State Public Health Surveillance Service;
5. to develop and approve mechanisms for adequate and sustainable financing from different sources (public health fees, National Health Insurance Company, national and local budget) for implementing public health programs;
6. to increase the funds meant for preventing and promoting population health so as to cover the needs for implementing the National Public Health Strategy;
7. to develop the legislative and normative framework for involving the National Health Insurance Company's prophylaxis fund and using the means from this fund exclusively for health protection, disease prevention, and health promotion activities;
8. to increase by 2020 the financing share for public health services, national programs for disease prevention and health promotion from the global financing share for health;
9. to monitor, evaluate, and publically communicate about the use of funds meant for prevention and promotion of public health.

Section 9. Scientific research in public health

112. Problems identified in scientific research area:

1. scientific research is carried out on fragmentary basis, without systemic vision of the priority health problems;
2. the financing of the research projects is insufficient, and the financing of institutions projects is carried out without taking into consideration the performance of the institutions and the public health priorities;
3. the role of the Ministry of Health in prioritizing the problems for research and development in the area is insufficient, it doesn't correspond to real situation in this area;
4. there are limited human and technological resources for carrying out scientific research and promoting results, including for participation in big international projects;
5. there is no financial model, which would foster competition, quality development, and visibility of scientific outputs, including the use of scientific achievement in practice;

6. there is insufficient coordination and collaboration among the national research institutions in health area, and this limits the real and efficient use of projects' outcomes;
7. the medical scientific community does not fully use the opportunities emerged after joining the EU Framework Program.

113. Specific objective: to consolidate research in public health area so as to ensure evidence-based reasoning for policies.

114. The following tasks are set forth to fulfill the specific objective:

1. to strengthen the technical-material base in compliance with the modern requirements for scientific research, including development of new research methods and innovation technologies;
2. to develop and use efficiently the scientific and technologic potential;
3. to extend collaboration scientific relations by establishing partnerships with domestic and overseas scientific research centers and institutions for carrying out scientific research in public health area.

115. Strategic interventions to be carried out:

1. to increase investments in the area of national public health science and innovation coming from the state, as well as to attract investments for carrying out research through active participation in competitions for fundamental and applicative research projects;
2. to increase the role of the Ministry of Health to prioritize the scientific problems which need urgent solutions, guide the scientific institutions towards implementation of complex projects related to up-to-date problems in public health area;
3. to consolidate relations with the partners from the EU and CIS area, to harness the existing partnerships for developing project proposals to participate in competitions under the EU Framework Program in public health area;
4. to get integrated in the international movement based on cooperation and specialization principles, in line with the priorities of the national public health policy.
5. to motivate the scientists to publish the results of the scientific research in international magazines with high impact;
6. to communicate the results of the scientific research regarding the health determinants and their impact on population health and risk factors to the interested authorities, population, and international organizations.



V. ESTIMATION OF IMPACT AND COSTS

116. To implement the Strategy, every responsible authority shall plan financial resources for the given purpose within the limits of the annually approved allocations for the budgets of the respective authorities. At the same time, the Ministry of Health shall participate in carrying out the Strategy as a coordinating, executive, and consultative body.

117. The technical and human resources necessary for implementing the Strategy are estimated and detailed for every stage of the implementation process in the Action Plan for implementing the Strategy (Annex No. 1 to the Strategy). The financial resources necessary for implementation of planned activities and actions are included in the Annex No. 2. The Action Plan will serve as well as the main mechanism for implementing into practice the Strategy and it will set the specific goals in the identified areas and the actions for achieving such goals, the expected results/benefits, the deadlines, the responsible institutions, and the indicators for assessing the obtained results (Annex No. 3 to the Strategy).



VI. MONITORING, EVALUATION, AND REPORTING

118. Monitoring of the Strategy implementation will be done by the Ministry of Health.

119. Evaluation of the Strategy implementation will be done at all stages:

1. during the development of intervention programs;
2. during the implementation of these programs for their restructuring, if needed;
3. after completion of the programs.

120. Evaluation will be carried out based on progress and performance indicators established for the Strategy.



VII. IMPLEMENTATION STAGES AND DEADLINES

121. This Strategy envisages a complex approach to fulfill the measures aiming to support the interactions between the Government and the society for improving significantly population health and wellbeing, reducing inequalities in health areas, and enhancing public health. It is planned to be implemented in two stages:

1) stage I – 2014 - 2016 – will mainly focus on improving and harmonizing the existing normative framework, including development of new acts in the public health area in line with the EU and WHO requirements, as well as on building the capacities of the partners with duties and responsibilities related to carrying out public health operations, services, and activities, including the improvement of State Public Health Surveillance Service operation;

2) stage II – 2017-2020 – will focus on implementing the provisions of legal and normative acts, carrying out actions to improve population health, hence contributing to reducing inequalities in health.

122. The deadlines for fulfilling the measures for developing, implementing, and assessing the actions related to capacity building and service consolidation in public health area are stipulated in the Action Plan for Strategy implementation, according to Annex No.1 to the document.



VIII. ROLES AND RESPONSIBILITIES OF PARTNERS

123. The structures involved in population health surveillance and wellbeing shall be fully accountable to the public. These structures shall provide public access to their strategic plans, evaluation results, obtained outcomes, information on usage of funds, and efficiency of institution activities.



IX. INTERNATIONAL COOPERATION

124. International cooperation shall develop in ascending way. The collaboration with traditional partners (WHO, UNICEF, UNFPA, GAVI, WB, AIEA, FAO, Codex Alimentarius, NATO, etc.) shall continue, as well as collaboration with interested countries, institutions, and services at the European level.

ANEX NO. 1

TO THE NATIONAL PUBLIC HEALTH STRATEGY 2014-2020

PLAN OF ACTIONS to implement the National Public Health Strategy for 2014-2020

No.	Actions	Deadlines	Responsible	Monitoring indicators
1	2	3	4	5
1. Specific Objective: To make more efficient and to consolidate the population health surveillance systems so as to identify the health problems and to supply relevant, accurate, and timely information for decisions and actions in public health area.				
1.1. Task: To enhance and increase the efficiency of the surveillance system for diseases and population health indicators.				
1.1.1	Revise the list of health indicators which are used and reported by the medical institutions and other institutions; establish definitions, characteristics, and clear categories for these indicators so as to align them to the international requirements	2014	Ministry of Health National Bureau of Statistics	1) Report on evaluation of health indicators 2) Approved list of health indicators with set definitions, characteristics, and categories
1.1.2	Develop amendments to the Law on Compulsory Health Insurance Funds for 2013 No. 251 dated 08.11.2012, amended via the Law No. 80 dated 18.04.2013, to ensure the financing of the activities for consolidating the infrastructure and the capacities of the health sector, prophylaxis and prevention of sickness risks, health promotion	2014	National Health Insurance Company, Ministry of Health	Developed and approved legislative act
1.1.3	Develop, test, and implement the system for data quality management	2017-2020	Ministry of Health	1) Approved concept for the data quality management system 2) Evaluation report for system testing 3) 60% of medical institutions apply the system of data quality management

1	2	3	4	5
1.1.4.	Evaluate the system of epidemiological surveillance and control of communicable diseases	2014	Ministry of Health	Evaluation report for the system for epidemiological surveillance and control of communicable diseases
1.1.5.	Delimit the functions and responsibilities of the institutions from the network of communicable diseases' control and epidemiological surveillance so as to adjust it to the EU requirements	2015	Ministry of Health	Order of the Ministry Of Health on functions and duties of the institutions for communicable diseases' control and epidemiological surveillance
1.1.6.	Adapt and integrate vertical systems for monitoring TB, HIV/ AIDS and STDs in the national system of communicable diseases' control and supervision	2018	Ministry of Health	Report on implementation of the integrated system for monitoring communicable diseases, including TB, HIV/AIDS and STDs
1.1.7.	Develop, test, and implement the system for monitoring microbiological and chemical contamination of foodstuff	2017	Ministry of Agriculture and Food Industry	1) Approved system concept 2) Evaluation report for system testing
1.1.8.	Integrate the system for monitoring microbiological and chemical contamination of foodstuff into the national surveillance system for priority communicable and non-communicable diseases, public health risks and emergencies	2020	Ministry of Agriculture and Food Industry Ministry of Health	Report on implementation of the System for monitoring microbiological and chemical contamination of foodstuff integrated into the national surveillance system for priority communicable and non-communicable diseases, public health risks and emergencies
1.1.9.	Evaluate the enforcement of the nutritional surveillance system in the pilot districts	2014	Ministry of Health	Evaluation report on implementation of the nutritional surveillance system in the pilot districts
1.1.10.	Implementation of the nutritional surveillance system at the national level	2016	Ministry of Health National Health Insurance Company	Evaluation report on the implementation of the nutritional surveillance system at the national level
1.1.11.	Evaluation of the "Cancer Register"	2014	Ministry of Health	Evaluation report on "Cancer Register"
1.1.12.	Development and implementation of the soft for "Cancer Register" which would ensure the link with the National Register of Population, demographic data and death causes	2018	Ministry of Health Ministry of Information Technology and Communication, National Health Insurance Company	"Cancer Register" applied in practice and its integration with the National Register

1	2	3	4	5
1.1.13.	Development, testing, and implementation of the surveillance system for non-communicable diseases and risk factors	2017-2018	Ministry of Health	1) Approved concept of the system 2) Evaluation report on the surveillance system for non-communicable diseases
1.1.14.	Development and enforcement of the legal framework on access of data and exchange of information in public health area between the relevant national structures	2014-2016	Ministry of Health, Ministry of Information Technology and Communications, Ministry of Agriculture, Ministry of Labor, Social Protection, and Family National Bureau of Statistics	Government Decision on access to data and exchange of information between the relevant national structures approved
1.2. Task: To monitor and periodically evaluate population health status, communicating the obtained results to interested authorities and population				
1.2.1.	Development of materials and training of specialists responsible for surveillance of communicable and non-communicable diseases and risk factors at the district/ regional and national levels regarding the epidemiological, surveillance and bio-statistical methods in public health	2015 - 2020	Ministry of Health	1. Developed materials 2. No. of trained specialists 3. Evaluation report of specialists' knowledge and practices.
1.2.2.	Continuous monitoring and systematic evaluation of the population health status, health determinants at the national and district/regional levels	2014-2020	Ministry of Health	Periodical evaluation reports on population health status published at the national district/ regional levels
1.2.3	Periodical surveillance and evaluation of the risk factors for communicable and non-communicable diseases at the national/regional levels through studies	2014 2020	Ministry of Health	Reports of the studies carried out at the national/ regional levels
1.2.4	Participation in international and regional studies on surveillance of health status and risk factors	2014-2020	Ministry of Health	Reports of the studies carried out at the national level
1. Specific Objective: To strengthen the national system for prevention, preparedness and response in public health emergencies through integrated approach to threats				
2.1. Task: Strengthening the national capacities for early detection and rapid response to public health threats and emergencies based on risk monitoring indicators at local, intermediary (district/municipality) and national levels.				
2.1.1	Evaluation of national capacities for early identification of threats and risks for health and rapid response	2015	Ministry of Health Ministry of Internal Affairs Ministry of Agriculture and Food Industry Ministry of Environment	Evaluation report

1	2	3	4	5
2.1.2	Establishment of the surveillance system for public health events with delimitation of functions at the local, district (intermediary), and national levels	2016	Ministry of Health Ministry of Internal Affairs	Approved joint Order on surveillance system for public health events
2.1.3.	Implementation and evaluation of the public health events' surveillance system	2018- 2020	Ministry of Health Ministry of Internal Affairs Ministry of Agriculture and Food Industry, Ministry of Environment in collaboration with local public authorities	1) Approved standard operation procedures 2) Approved joint evaluation report at the meeting of the National Extraordinary Commission of Public Health
2.1.4	Periodical evaluation of the early warning system for prevention and control of communicable diseases and public health events	2014 - 2020	Ministry of Health Ministry of Agriculture and Food Industry, Ministry of Environment in collaboration with local public authorities	Reports on results of the simulation exercises
2.1.5	Adjustment of the operational procedures of the authorities involved in the early warning and rapid response system for prevention and control of communicable diseases and public health events	2014-2020	Ministry of Health Ministry of Agriculture and Food Industry, Ministry of Environment in collaboration with local public authorities	Operational procedures adjusted and approved by the involved authorities
2.1.6	Adjustment of the legal framework (regulations, standards, guides, indications and methodic recommendations) on public health emergencies of chemical, biological, radiological origin to the international standards	2014-2020	Ministry of Health Ministry of Internal Affairs, Ministry of Agriculture and Food Industry, Ministry of Environment	Legislative acts developed and approved by the involved ministries
2.1.7	Establishment and functional assurance of the operational center for management of public health emergencies as part of the international networks, including for the implementation of the International Sanitary Regulation 2005, INFOSAN, ChemiNet, GladNet within the National Center of Public Health.	2016-2020	Ministry of Health	1) Order of the Ministry of Health 2) Evaluation report of the test exercises' results for center functionality

1	2	3	4	5
2.1.8	Establishment and implementation of the National Integrated System of health sector preparedness and response to public health emergencies	2015	Ministry of Health	1) Order of the Ministry of Health 2) Evaluation report of the test exercises' results for system functionality
2.1.9	Evaluation of the lab network capacities under the public health surveillance service and efficiency of the response in public health emergencies	2014	Ministry of Health	Evaluation report using the WHO evaluation questionnaire (LAT)
2.1.10	Strengthen and maintain the lab network under the public health surveillance service for increasing the level of confirmation of threats of biological, chemical, and radiological origin	2014-2020	Ministry of Health	1) Share of performance labs equipped according to the minimum set of devices 2) Annual evaluation reports for the confirmation level
2.1.11	Equipping the national and territorial intervention teams in case of public health risks and emergencies	2014-2020	Ministry of Health	100% national and territorial teams equipped in line with the unified list of equipping
2.1.12	Periodical training of the intervention teams on planning, preparing, estimating the needs and vulnerabilities, evaluating, managing and communicating the risks, and post-intervention evaluation	2014 - 2020	Ministry of Health	Periodical evaluation reports
2.1.13	Establishment and operational assurance of the National Committee on Biosecurity and Biosafety of labs that work with microbial agents and toxins	2015	Information and Security Service Ministry of Defense Ministry of Health Ministry of Agriculture and Food Industry, Academy of Science	Decision on establishment, approval of the Committee Activity Regulation and the National Register of Labs
2.1.14	Establishment and maintenance of the National Register of Labs that work with microbial agents and toxins for ensuring biosafety and biosecurity	2015-2020	Ministry of Health Ministry of Agriculture and Food Industry	1) Ministry of Health Order on administration of the Register 2) National Register of Labs with data security system created and launched into operation
2.1.15	Development of mechanisms for planning and distributing financial resources for carrying out public health emergency activities	2015	Ministry of Health National Health Insurance Company Ministry of Finance	1) Order on mechanisms for mobilization and distribution of financial resources for carrying out public health emergency activities 2) Evaluation report for the resource mobilization mechanism

1	2	3	4	5
2.2. Task: Minimizing the negative impact on population health, determined by public health risks and emergencies caused by natural, technogenic, biological, radiological and social factors.				
2.2.1	Mapping the threats (natural, technogenic, biological, radiological and social) for public health	2015	Ministry of Health, Ministry of Environment, Ministry of Agriculture and Food Industry, Ministry of Internal Affairs, Ministry of Defense	List of natural, technogenic, biological, radiological and social threats finalized and approved
2.2.2	Development of complex plans for preparedness and response to public health emergencies; testing and revising them within a predefined periodicity	2014; 2019	Ministry of Health, Ministry of Internal Affairs, Ministry of Agriculture and Food Industry, Ministry of Environment, in collaboration with local public authorities	1) Plan approved 2) Reports on Plan testing
2.2.3	Establishment and operational assurance of the Toxicological Safety Center within the NPHC for early detection and rapid response to public health events of chemical origin	2016 - 2020	Ministry of Health	1) Order of the Ministry of Health 2) Report on Center's operation testing
2.2.4	Integration into regional and global networks of surveillance and response: Global Alert and Response (GAR), Global Chemical Incident Emergency Response Network (ChemiNet), International Food Safety Authorities Network (IFOSAN), etc.	2014-2020	Ministry of Health	Evaluation of the integration progress (no. of networks).
2. Specific Objective: To ensure health protection by making more efficient the control over behavioral and environmental risk factors				
3.1. Task: To improve the legal and regulatory framework in health protection area by harmonizing it with the international regulations.				
3.1.1.	Modification and harmonization of the national legislative framework with the international regulations which involves: - tobacco control; - alcohol control; - foodstuff ; - public health surveillance.	2014-2018	Ministry of Health, Ministry of Labor, Social Protection, and Family, Ministry of Agriculture and Food Industry	Legislative acts modified and approved

1	2	3	4	5
3.1.2.	Development of the regulation framework harmonized with the EU standards and WHO recommendations: • Sanitary regulation on internal air quality; • Sanitary regulation on quality of air at work; • Sanitary regulation on cosmetics; • Sanitary regulation on sanitary protection zones for water sources; • Sanitary regulation on surveillance of drinking water quality; • Sanitary regulation on management of waste from medical activity; • Sanitary regulation on operation of community-based social services designed for persons with disabilities	2014-2016	Ministry of Health, Ministry of Regional Development and Constructions, Ministry of Labor, Social Protection, and Family, Ministry of Agriculture and Food Industry	Normative acts developed and approved
3.1.3	Development of national policy documents and actions for improving environmental factors and reducing health risks, including: • National Action Plan on Food and Nutrition; • Action Plan for implementing the Protocol on Water and Health; • Action Plan for environmental health, according to the commitments assumed at Parma in 2010; • Sector Strategy and Action Plan on adjusting to extreme phenomena of climate changes and guidelines for enforcing them. • National Strategy for combating antimicrobial resistance	2014-2017	Ministry of Health, Ministry of Environment	Policy documents developed and approved

1	2	3	4	5
3.2. Task: To enhance the State Public Health Surveillance Service capacities to assess, manage, and communicate the risks for public health and to establish the mechanism for collaboration with other services in the area.				
3.2.1	Increasing capacities for identifying and monitoring risks in health protection area through: • Endowment with equipment; • Strengthening human capacities; • Implementation of new investigation methods and diversification of public health services offer • Financial coverage of the labs' needs for ensuring the operation of the public health protection system via diversification of financing sources.	2014-2016	Ministry of Health, National Health Insurance Company	1. Share of accredited labs 2. Number of implemented new methods and services 3. Level of financial coverage (%) of the needs
3.2.2	Evaluation of labs' activities with relevance for public health area, regardless of the ownership form, with development of their register and appointment of reference labs	2014-2016	Ministry of Health Ministry of Agriculture and Food Industry, in collaboration with the private sector	1. Act for approving the Register 2. Register created and launched into operation
3.2.3	Revision of the activity regulations of the State Public Health Surveillance Service subdivisions at the national and local levels involved in health protection area	2014-2016	Ministry of Health	Regulations approved
3.2.4	Implementation of good practices for patients' safety (regulations of good practices in healthcare, injections' safety, management of wastes resulting from medical activity, etc.).	2014-2020	Ministry of Health, National Health Insurance Company	% of institutions accredited in line with standards of good practices
3.2.5	Establishment of an information system for environmental health and behavior risk factors based on the indicators recommended by the WHO.	2016	Ministry of Health Ministry of Environment	Information system established and operational
3.2.6	Development, testing, and implementation of the surveillance system for occupation health	2018	Ministry of Health Ministry of Labor, Social Protection and Family	1) Concept of the system approved 2) Evaluation report on testing the surveillance system for occupational health

1	2	3	4	5
3.2.7	Consolidation of institutional capacities of the State Public Health Surveillance Service in the area of occupational health and working environment by creating the respective subdivisions.	2015	Ministry of Health, Ministry of Labor, Social Protection and Family	Subdivisions created
3.2.8	Creation and maintenance of the National Register of Chemicals for detection of health emergencies of chemical origin	2014-2015	Ministry of Health, Ministry of Environment, Ministry of Labor, Social Protection and Family	National Register with data security system created and launched into operation
3.3. Task: To strengthen the capacities for identifying and managing the public health risks for other authorities with duties in the area of health protection (environmental health, occupational health, food safety).				
3.3.1	Development of the mechanism for reporting environmental health indicators	2016	Ministry of Health Ministry of Environment	Ministry of Health Order issued
3.3.2.	Development of efficient inter-sector communication mechanisms in health protection area	2015	Ministry of Health, Ministry of Agriculture and Food Industry, Ministry of Environment, Ministry of Labor, Social Protection and Family	Joint plans/agreements of activity developed and approved
4. Specific Objective: To adopt some healthy behaviors among the population of the Republic of Moldova by implementing efficient and coordinate measures for health promotion by the different sectors at the national and local levels.				
4.1. Task: To consolidate the health sector's infrastructure and capacities in health promotion area				
4.1.1	Evaluation and update of the National Program on Health Promotion;	2016	Ministry of Health	Program updated
4.1.2	Allocation of financial resources from the state and local budgets for health promotion in a necessary volume to implement national and territorial programs	2014-2020	Ministry of Finance Ministry of Health, in collaboration with local public authorities	Volume of allocated budget
4.1.3	Management of the National Health Insurance Company prophylaxis fund and reserve fund depending on the priorities stated by the Ministry of Health based on the argumentation submitted by the State Public Health Surveillance Service	2014	Ministry of Health, National Health Insurance Company, Ministry of Finance	Developed regulation
4.1.4.	Development of activity regulations and guidelines for health promotion for the specialists from public health, primary healthcare, etc.	2016	Ministry of Health	Developed regulations and guidelines

1	2	3	4	5
4.1.5.	Revision of the job descriptions of the specialists from the health system so as to include the responsibilities for health promotion in line with the European standards	2015	Ministry of Health	Revised and approved job descriptions
4.1.6.	Establishment of performance indicators which include activities for health promotion	2015	Ministry of Health National Health Insurance Company	Indicators established
4.1.7	Establishment of the personnel unit responsible for health promotion within the primary healthcare	2014	Ministry of Health, National Health Insurance Company	Personnel unit established
4.1.8	Development of communication skills for health promotion among health workers, teaching personnel, youth, etc.	2014-2020	Ministry of Health Ministry of Education Ministry of Youth and Sport, Ministry of Labor, Social Protection and Family in collaboration with profile NGOs, international organizations	Number of trained personnel
4.1.9	Development and implementation of plans for population information and education at the community level in the context of behavior change and cultivation of healthy habits for preventing communicable and non-communicable diseases.	2014-2020	Ministry of Health, Ministry of Education, Ministry of Youth and Sport, Ministry of Labor, Social Protection and Family in collaboration with profile NGOs, international organizations	Communication plans developed and implemented
4.1.10	Development of the methodological support for teaching personnel for training the youth and children in the area of health promotion and disease prophylaxis.	2014-2015	Ministry of Health Ministry of Education	Number of guides developed
4.1.11	Development of advocacy campaigns for implementing priority measures of public health (combating tobacco and alcohol consumption, promotion of immunization, etc.)	2014-2020	Ministry of Health in collaboration with the Consumer Protection Agency and mass-media	Number of implemented campaigns

1	2	3	4	5
4.2. Task: To increase the potential and involvement of other sectors than health sector in activities for health promotion and improvement of inter-sector collaboration at the national, territorial/ local levels.				
4.2.1.	Establishment of inter-sector groups for carrying out health promotion activities	2014	Ministry of Labor, Social Protection and Family, Ministry of Education, Ministry of Environment, Ministry of Youth and Sport, Ministry of Health, Mass media, in collaboration with profile NGOs	Groups established
4.2.2.	Develop guidelines and informational materials for professionals involved in health promotion activities (local public authorities, other sectors)	2015 - 2018	Ministry of Health	Guides and materials developed
4.3. Task: To create health environments by establishing partnerships with civil society and empowering communities to plan and implement efficient health promotions activities.				
4.3.1	Implementation of WHO initiatives and programs, such as: - "health promotion schools", - "healthy hospitals" - "healthy cities" - "healthy localities"	2014-2018	Ministry of Health, Ministry of Education, Ministry of Youth and Sport, Ministry of Labor, Social Protection and Family in collaboration with local public authorities, profile NGOs, and international organizations	Number of implemented programs
4.3.2.	Development and approval of criteria for healthy environments/ communities by piloting "localities' health profiles"	2015-2017	Ministry of Health Ministry of Environment, Ministry of Education	Criteria developed and approved
4.3.3	Mapping and consolidation of sustainable partnerships with NGOs and civil society in carrying out activities for health promotion	2014-2015	Ministry of Health in collaboration with local public authorities and profile NGOs	Number of partner institutions and NGOs

1	2	3	4	5
4.3.4	Carrying out information and communication campaigns about the behavioral risk factors for different target groups	2014-2020	Ministry of Health, National Health Insurance Company, Ministry of Education, Ministry of Youth and Sport, Ministry of Labor, Social Protection and Family, in collaboration with mass-media and profile NGOs	Number of carried out campaigns
5 Specific Objective: To reduce the burden of communicable and non-communicable diseases by reducing the risk factors and ensuring equitable access of population to primary, secondary, and tertiary prevention services				
5.1. Task: To decrease the risk factors at the individual, community, and national levels through integrated activities of health sector together with other sectors and local public authorities.				
5.1.1.	Development of national and territorial programs focused on activities for early detection of risk factors and pre-morbid conditions or incipient stages of diseases by maintaining and strengthening national surveillance and control systems for communicable and non-communicable diseases	2014-2020	Ministry of Health, Ministry of Agriculture and Food Industry, Ministry of Internal Affairs, Ministry of Environment, Ministry of Education, Ministry of Transport and Road Infrastructure, in collaboration with local public authorities	1) No. of approved national programs for disease prevention 2) 100% of administrative territories with approved territorial programs
5.1.2	Implementation of mechanisms for assessing the efficiency of programs meant to reduce risks and prevent diseases (financed per prevented case) for primary, hospital, and specialized healthcare	2015-2020	Ministry of Health, National Health Insurance Company	Mechanism developed and implemented
5.1.3	Fulfillment of the Action Plan for implementing the National Strategy for NCD control and prevention, according to the Plan provisions	2014-2020	Ministry of Health, interested ministries and agencies	Annual report*

1	2	3	4	5
5.1.4	Development of a set of indicators used by the State Public Health Surveillance Service, local public authorities for assessing population health condition as a result of disease prevention programs' implementation	2017	Ministry of Health	Developed set of indicators
5.1.5	Development of guidelines and protocols for medical workers with secondary and higher education for early identification of diseases, associated risk factors, and their control.	2014-2020	Ministry of Health	Number of developed guidelines and protocols
5.2. Task: To prevent and control vaccine-preventable diseases by covering the target groups with vaccines in line with the provisions of the national immunization programs and to decrease communicable diseases' morbidity by 2% by 2020 via measures of primary and secondary prevention				
5.2.1	Assessment of stewardship and international certification of the immunization system (at the level of primary healthcare, public health centers, NPHC, national storage of vaccines)	2014, 2018,	Ministry of Health	Obtained international certification
5.2.2	Establishment and operational assurance of the National Technical Consultative Committee in Immunization area	2014	Ministry of Health	Committee established and operational
5.2.3	Periodical assessment of the efficiency of the immunization program through sero-prevalence studies, including by estimating the indicators reflecting accessibility, equity, efficiency, including in vulnerable groups, such as migrants and ethnic minorities	2015-2020	Ministry of Health	Studies carried out, Report submitted
5.2.4	Securing financial resources for procuring, stocking, maintaining the "cold chain", depositing, transporting vaccines and consumables for the implementation of the vaccination programs, including the "call and recall" systems	2016-2020	Ministry of Finance Ministry of Health	100% financing of the envisaged volume

1	2	3	4	5
5.3. Task: To ensure equitable access to quality services of disease prevention provided by primary, hospital, and specialized health care and public health surveillance service.				
5.3.1	Development of programs for diseases prevention based on needs' evaluation of the risk and vulnerable groups (e.g.: PCV vaccination of persons aged 65+, vaccination against HVB of health workers, etc.)	2016-2020	Ministry of Health, Ministry of Labor, Social Protection and Family	Programs developed and implemented
5.3.2	Development and implementation of training programs for the specialists involved in preventing communicable and non-communicable diseases	2014-2015	Ministry of Health	Training curricula developed
5.3.3	Development and implementation of the counseling system (in general aspect, mental health, reproductive health, etc.) for preventing the risks and disease appearance	2015-2017	Ministry of Health, National Health Insurance Company, Ministry of Education	Concept of the system approved Financial resources defined for concept implementation
6. Specific Objective: To improve the cross-sector collaboration mechanisms with clear definition of responsibilities among the health sector and other sectors in implementing the essential public health operations				
6.1. Task: To strengthen collaboration between the health sector and other sectors so as to reduce the risk factors and improve population health status, applying the "health in all policies" principle				
6.1.1.	Development of cross-sector policies and programs, and establishment of the National Coordinating Council in public health area with clearly defined responsibilities for every stakeholder	2014-2016	Ministry of Health	National Council Developed and approved programs and policies
6.1.2.	Development and annual implementation of the cross-sector communication plan in public health area	2015-2016	Ministry of Health, interested ministries and services	Developed plans
6.1.3.	Training decision makers from central and local public authorities regarding the general public health problems and sector responsibilities for population and target groups' health	2015-2016	Ministry of Health, Academy of Public Administration under the President of the Republic of Moldova	Number of trained persons
6.1.4.	Establishment of the regulatory framework for ensuring State Public Health Surveillance Service coordinating role in the public health area at the territorial level, under the local public authorities	2014	Ministry of Health in collaboration with local public authorities	Normative act developed and approved

1	2	3	4	5
6.1.5.	Strengthening capacities of the Ministry of Health and subordinated profile institutions to assess and estimate the impact of other sectors' policies on population health status	2014-2020	Ministry of Health	Adequate capacities of the Ministry of Health and subordinated profile institutions
6.2. Task: To improve the quality of medical and public health services, efficiency of interventions to reduce inequity in health				
6.2.1	Development and implementation of mechanisms for planning means from the National Health Insurance Company fund for prophylaxis measures	2014-2015	Ministry of Health, National Health Insurance Company	Established Coordinating Council; Annual plan approved by the Council
6.2.2.	Periodical evaluation of implementation of policies and programs with impact on population health	2016-2020	Ministry of Health	Assessed periodical reports
6.2.3.	Development of standardized indicators of quality evaluation for public health services	2016-2020	Ministry of Health	List of indicators
6.2.4.	Competition-based selection of public health managers	2015	Ministry of Health	Number of persons selected on competition basis
6.2.5.	Development of partnerships at the national and local levels with involvement of non-governmental organizations and community	2014-2015	Ministry of Health, interested ministries and services, in collaboration with local public authorities, profile NGOs	Partnership agreements signed at national and local levels
6.3. Task: To implement e-governance mechanisms in public health services.				
6.3.1.	Unifying information flows in the health system	2014-2015	Ministry of Health, National Health Insurance Company	Integrated information system in health
6.3.2.	Aligning the public health information system to the National Digitization Strategy	2015-2016	Ministry of Health	Ministry of Health Order
6.3.3.	Adapting quality standards and common definitions to European rules (WHO and EUROSTAT)	2014-2015	Ministry of Health	Joint Order
6.3.4.	Revision of the public health indicators in line with the requirements of the WHO and EU reports	2014-2015	Ministry of Health	Set of indicators
6.3.5.	Improvement of dissemination of public health information through modern means	2015	Ministry of Health	Ministry of Health Order
6.3.6.	Continuous training of specialists involved in public health information management	2014	Ministry of Health	Ministry of Health Order to approve continuous training programs

1	2	3	4	5
7. Specific Objective: To ensure with sufficient and competent human resources in public health area by strengthening the training system for undertaking the essential public health operations.				
7.1. Task: To staff the public health system structures with sufficient number of qualified personnel and to enhance the legislative framework; to estimate the needs and to plan human resources for carrying out public health essential operations.				
7.1.1	Evaluation of needs of the specialized providers of public health services, of the scope, roles, and responsibilities for carrying out the new tasks of public health and adjusting the necessary number of specialists for talking them.	2014	Ministry of Health	Norms developed and applied
7.1.2	Development, approval, and implementation of methodology for strategic planning of human resources in public health area.	2015	Ministry of Health	Approved strategic planning methodology
7.1.3.	Development of the educational legislative framework corresponding to the international standards for public health	2015-2017	Ministry of Health Ministry of Education	Approved normative acts
7.1.4.	Revision of professional training route in public health area at all the training stages: secondary specialized, undergraduate, graduate, postgraduate and continuous in line with the Bologna Process and within ASPHER for 3 groups of public health specialists: - specialists from public health area who work at a senior management level or with a high level of expertise; - professionals who dedicate a lot of their time to activities in public health area, such as clinicians, ecologists, etc.; - professionals who indirectly contribute to public health, including teachers, social assistants, etc.	2014-2016	Ministry of Health Ministry of Education	Approved concept for training of public health specialists
7.1.5.	Revision of the Nomenclature of specialties and Scientific Classifier for adjusting them to modern public health approaches	2014	Ministry of Health National Council for Accreditation and Attestation	Nomenclature updated

1	2	3	4	5
7.2. Task: To modernize the process and stages of professional training, adjusting the curricular components of the pre-university, undergraduate, postgraduate, and continuous training of the personnel in line with the essential competences for the public health set by the Association of Schools of Public Health in the European Region (ASPHER).				
7.2.1.	Development and implementation of plan of measure for implementing international requirements recommended by the ASPHER	2014-2020	Ministry of Health Ministry of Education	1) Plan developed and approved 2) International accreditation of public health schools
7.2.2.	Adjusting training programs to the international ones for facilitating the mobility of public health specialists with approval and implementation of such programs	2014-2020	Ministry of Health Ministry of Education	Programs and curricula adjusted to ASPHER standards and approved
7.2.3.	Revision of plans, training curricula and professional internships according to the capacities needed by the national public health system.	2014-2018	Ministry of Health Ministry of Education	Training/practical plans/curricula coordinated, adjusted, approved, and implemented.
7.2.4.	Implementation of new learning forms (distance learning, problem-based learning, etc.) in public health area	2014-2020	Ministry of Education, Ministry of Health, international partners	No. of persons trains via new learning methods
7.2.5.	Extending educational offers by strengthening the capacities of the National School of Management in Public Health	2014-2020	Ministry of Health in collaboration with international partners	No. of master degree and PHD programs
7.2.6.	Development and implementation of mechanisms for licensing specialists in public health area	2014-2015	Ministry of Health	Licensing mechanism adopted and implemented
7.2.7.	Legislating continuous training of non-medical specialists with competence in public health area	2014-2017	Ministry of Health	Training concept for specialists in public health developed and approved
7.2.8.	Involvement of professional associations and civil society in professional development activities	2014-2020	Ministry of Health	Number of conferences, seminars, trainings
7.3. Task: To staff the public health system with human resources in a sufficient and balanced way by developing and implementing innovating programs for recruiting, maintaining, and developing the personnel				
7.3.1.	Development and implementation of professional performance-based payroll system	2014-2015	Ministry of Health	1. System developed 2. Implementation act
7.3.2.	Development and implementation of the payroll system for public health specialists to ensure an income level equivalent to the one in health sector	2014-2020	Ministry of Labor, Social Protection and Family, Ministry of Finance, Ministry of Health	Mechanism developed and approved
7.3.3.	Ensuring adequate working conditions for the specialists in public health area	2017	Ministry of Health Ministry of Finance	Regulation developed and approved

1	2	3	4	5
7.3.4.	Revision of legal framework for granting facilities to young specialists in the area	2015-2020	Ministry of Health	Revised legislative-normative framework
7.3.5.	Development and approval of methodology for professional promotion based on objective criteria of merit (on competition basis)	2015	Ministry of Health	Methodology approved
7.3.6	Development of national and international partnerships, networks and consortiums between the specialists from education and public health areas	Over the time	Ministry of Health Ministry of Education	Number of fulfilled collaboration agreements, educational programs.
8. Specific Objective: To adjust the organizational structure and to improve the financing of the State Public Health Surveillance Service so as to carry out the essential operations and services in public health area in collaboration with other sectors				
8.1. Task: To consolidate the State Public Health Surveillance Service and other health services at the national and territorial levels for ensuring the fulfillment of essential public health operations and services.				
8.1.1.	Development and promotion of the Concept on State Public Health Surveillance Service regionalization, including of the lab network.	2014-2015	Ministry of Health	Concept developed
8.1.2.	Development and approval of a set of normative acts about the implementation of the provisions under the Concept on State Public Health Surveillance Service regionalization, including of the lab network	2014-2015	Ministry of Health	Ministry of Health Orders
8.1.3.	Training of the personnel of territorial Public Health Centers regarding the State Public Health Surveillance Service regionalization, including of the lab network.	2014-2015	Ministry of Health	Workshops
8.1.4.	Establishment of the Public Health Performance Centers, including labs, their technical-material equipping with modern devices and techniques.	2014-2017	Ministry of Health	Performance centers and labs established
8.1.5.	Evaluation and improvement of the Public Health Performance Centers' activity	2015-2020	Ministry of Health	Evaluation reports

1	2	3	4	5
8.1.7.	Reorganization of the rayon Public Health Centers which did not become Performance Centers, re-profiling the personnel remaining in the territory and locating them based on the territorial public medical-sanitary institutions or based on other institutions (as the case may be).	2016	Ministry of Health	Ministry of Health Orders
8.2. Task: To ensure sustainable financing for the State Public Health Surveillance Service by implementing new financing mechanisms for fulfilling public health functions and essential activities.				
8.2.1.	Increasing the funds meant for prevention and promotion of population health by ensuring the implementation of the National Public Health Strategy.	2014-2020	Ministry of Health, National Health Insurance Company, Interested ministries and services	Volume of financial allocations
8.2.2.	Ensuring sustainable financing for carrying out national programs of public health and health promotion.	2014-2020	Ministry of Health, Ministry of Finance, National Health Insurance Company	Volume of financial allocations
8.2.3.	Identification and development of the mechanisms to sustainable mobilize the financial resources for implementing the national programs for disease prevention and control	2014-2016	Ministry of Health Ministry of Finance	% increase of the financing for the national programs for diseases prevention and control
8.2.4.	Development and approval of the Regulation on use of the Prophylaxis Fund and Reserve Fund of the National Health Insurance Company.	2014	Ministry of Health, National Health Insurance Company	Approved regulation
8.2.5.	Increasing the financing share for public health services and programs out of the total budget for health	2014-2020	Ministry of Health, National Health Insurance Company	Volume of financial allocations according to the Budget Law
8.2.6.	Cost-efficiency evaluation of the health protection, prevention, and promotion measures	2014-2016	Ministry of Health, National Health Insurance Company, interested ministries and agencies	Evaluation reports
8.2.7.	Monitoring, evaluation, and public communication about the use of funds designated for population health protection, prevention, and promotion.	2014-2020	Ministry of Health, National Health Insurance Company, interested ministries and agencies	Reports published

1	2	3	4	5
9. Specific Objective: To consolidate research in the area of public health so as to ensure evidence-based reasoning for policies				
9.1 Task: To strengthen the technical-material base in compliance with the modern requirements for scientific research, including developing new research methods and innovation technologies.				
9.1.1	Development of the technical-material basis for research-development by ensuring the organizations from science and innovation in public health area	2014-2020	Moldova Academy of Science Ministry of Health, National Health Insurance Company	Devices and equipment procured
9.1.2	Expertise of the research programs, projects, and results in public health area.	2014-2020	Ministry of Health, Moldova Academy of Science	Number of annual and final research projects and reports subject to expertise
9.1.3	Development and implementation of a financing model that would foster competition, development and enforcement of scientific results in public health.	2014-2015	Ministry of Health, Academy of Science of Moldova	Regulation developed
9.2. Task: To develop and use efficiently the scientific and technologic potential				
9.2.1.	Training highly qualified scientific personnel in the area of public health, fostering internships abroad.	2014-2020	Ministry of Health, Moldova Academy of Science	1) Number of highly qualified personnel; 2) Trainings in profile institutions abroad
9.2.2	Development of mechanisms for increasing material motivation of scientists	2014-2017	Ministry of Health, Moldova Academy of Science	Joint Order of the Ministry of Health and Moldova Academy of Science
9.2.3	Increasing the volume, clarity and visibility of scientific production	2014-2020	Ministry of Health, Moldova Academy of Science , AGEPI	Number of publications; patents; author certificates
9.3. Task: To extend collaboration scientific relations by establishing partnerships with domestic and overseas scientific research centers and institutions for carrying out scientific research in public health area.				
9.3.1	Creation of a single partnership of the national scientific institutions for developing and implementing research projects in the area of public health, focused on solving priority problems.	2014-2020	Ministry of Health, Moldova Academy of Science, Ministry of Environment, Ministry of Agriculture and Food Industry, Ministry of Finance, and Ministry of Labor, Social Protection and Family	Number of projects
9.3.2	Integration in the international research space by participating in international research programs	2014-2020	Ministry of Health, Moldova Academy of Science	Number of projects
9.3.3	Increasing visibility of the scientific achievements at the national and international levels and ensuring availability of the research-development activities' results.	2014-2020	Ministry of Health, Moldova Academy of Science	1) Number of scientific articles published, including ISI; 2) Reports published

ANEX NO. 2

OF THE NATIONAL PUBLIC HEALTH STRATEGY FOR 2014-2020

Financing the actions for implementing the National Public Health Strategy for 2014-2020

No.	Planned actions	Financing, thousand lei				
		Total	including from			
			State budget	Compulsory Health Insurance Funds	Projects and grants	Uncovered costs
1	2	3	4	5	6	7
Year 2014						
1.	Enhancing and increasing the efficiency of the surveillance system for diseases and population health indicators.	1050.0	300,0	300,0	450,0	0
2.	Monitoring and periodically evaluating population health status, communicating the obtained results to interested authorities and population	600.0	300,0	0	300,0	0
3.	Strengthening the national capacities for early detection and rapid response to public health threats and emergencies based on risk monitoring indicators at local, intermediary (district/municipality) and national levels	780,0	480,0	0	300,0	0
4.	Minimizing the negative impact on population health, determined by public health risks and emergencies caused by natural, technogenic, biological, radiological and social factors	0	0	0	0	0
5.	Improving the legal and regulatory framework in health protection area by harmonizing it with the international regulations	260,0	120,0	0	140,0	0

1	2	3	4	5	6	7
6.	Enhancing the State Public Health Surveillance Service capacities to assess, manage, and communicate the risks for public health and to establish the mechanism for collaboration with other services in the area	61700,0	3025,0	1000,0	57675,0	0
7.	Consolidating the health sector's infrastructure and capacities in health promotion area	37450,0	350,0	36750,0	300,0	50,0
8.	Increasing the potential and involvement of other sectors than health sector in activities for health promotion and improvement of inter-sector collaboration at the national, and territorial/local levels	0	0	0	0	0
9.	Creating health environments by establishing partnerships with civil society and empowering communities to plan and implement efficient health promotions activities	900,0	200,0	0	700,0	0
10.	Decreasing the risk factors at the individual, community, and national levels through integrated activities of health sector together with other sectors and local public authorities	250,0	150,0	0	100,0	0
11.	Preventing and controlling vaccine-preventable diseases by covering the target groups with vaccines in line with the provisions of the national immunization programs and to decrease communicable diseases' morbidity by 2% by 2020 via measures of primary and secondary prevention	250,0	100	0	150,0	0
12.	Ensuring equitable access to quality services of disease prevention provided by primary, hospital, and specialized health care and public health surveillance service	0	0	0	0	0
13.	Strengthening collaboration between the health sector and other sectors so as to reduce the risk factors and improve population health status, applying the principle "health in all policies"	1240,0	110,0	50,0	30,0	1050,0
14.	Improving the quality of medical and public health services, efficiency of interventions to reduce inequity in health	34,5	9,5	0	25,0	0
15.	Implementing e-governance mechanisms in public health services	5415,0	115,0	3300,0	300,0	1700,0
16.	Staffing the public health system structures with sufficient number of qualified personnel and to enhance the legislative framework; to estimate the needs and to plan human resources for carrying out public health essential operations	40,0	30,0	0	10,0	0
17.	Modernizing the process and stages of professional training, adjusting the curricular components of the pre-university, undergraduate, postgraduate, and continuous training of the personnel in line with the essential competences for the public health set by the Association of Schools of Public Health in the European Region (ASPHER)	0	0	0	0	0

1	2	3	4	5	6	7
18.	Staffing the public health system with human resources in a sufficient and balanced way by developing and implementing innovating programs for recruiting, maintaining, and developing the personnel	6000,0	6000,0	0	0	0
19.	Consolidating the State Public Health Surveillance Service and other health services at the national and territorial levels for ensuring the fulfillment of essential public health operations and services.	120,0	120,0	0	0	0
20.	Strengthening the technical-material base in compliance with the modern requirements for scientific research, including developing new research methods and innovation technologies	5000,0	2500,0	250,0	2000,0	250,0
21.	Developing and using efficiently the scientific and technological potential	1000,0	600,0	0	200,0	200,0
22.	Extending collaboration scientific relations by establishing partnerships with domestic and overseas scientific research centers and institutions for carrying out scientific research in public health area	500,0	200,0	0	200,0	100,0
Total		122589,5	14709,5	41650,0	62880,0	3350,0
Year 2015						
1.	Enhancing and increasing the efficiency of the surveillance system for diseases and population health indicators.	600,0	200,0	100,0	300,0	0
2.	Monitoring and periodically evaluating population health status, communicating the obtained results to interested authorities and population	600,0	200,0	0	400,0	0
3.	Strengthening the national capacities for early detection and rapid response to public health threats and emergencies based on risk monitoring indicators at local, intermediary (district/municipality) and national levels	1150,0	800,0	0	350,0	0
4.	Minimizing the negative impact on population health, determined by public health risks and emergencies caused by natural, technogenic, biological, radiological and social factors	0	0	0	0	0
5.	Improving the legal and regulatory framework in health protection area by harmonizing it with the international regulations	260,0	120,0	0	140,0	0
6.	Enhancing the State Public Health Surveillance Service capacities to assess, manage, and communicate the risks for public health and to establish the mechanism for collaboration with other services in the area	61800,0	8800,0	3000,0	50000,0	0
7.	Consolidating the health sector's infrastructure and capacities in health promotion area	1450,0	20,0	1100,0	300,0	30,0

1	2	3	4	5	6	7
8.	Increasing the potential and involvement of other sectors than health sector in activities for health promotion and improvement of inter-sector collaboration at the national, and territorial/local levels	1250,0	125,0	1125,0	0	0
9.	Creating health environments by establishing partnerships with civil society and empowering communities to plan and implement efficient health promotions activities	1400,0	200,0	500,0	700,0	0
10.	Decreasing the risk factors at the individual, community, and national levels through integrated activities of health sector together with other sectors and local public authorities	150,0	60,0	90,0	0	0
11.	Preventing and controlling vaccine-preventable diseases by covering the target groups with vaccines in line with the provisions of the national immunization programs and to decrease communicable diseases' morbidity by 2% by 2020 via measures of primary and secondary prevention	50,0	20,0	0	30,0	0
12.	Ensuring equitable access to quality services of disease prevention provided by primary, hospital, and specialized health care and public health surveillance service	4000,0	0	3500,0	500,0	0
13.	Strengthening collaboration between the health sector and other sectors so as to reduce the risk factors and improve population health status, applying the "health in all policies" principle	1166,0	251,0	35,0	30,0	850,0
14.	Improving the quality of medical and public health services, efficiency of interventions to reduce inequity in health	54,5	14,5	0	40,0	0
15.	Implementing e-governance mechanisms in public health services	7.055,0	1995,0	3000,0	350,0	1710,0
16.	Staffing the public health system structures with sufficient number of qualified personnel and to enhance the legislative framework; to estimate the needs and to plan human resources for carrying out public health essential operations	180,0	90,0	0	90,0	0
17.	Modernizing the process and stages of professional training, adjusting the curricular components of the pre-university, undergraduate, postgraduate, and continuous training of the personnel in line with the essential competences for the public health set by the Association of Schools of Public Health in the European Region (ASPHER)	0	0	0	0	0
18.	Staffing the public health system with human resources in a sufficient and balanced way by developing and implementing innovating programs for recruiting, maintaining, and developing the personnel	6000,0	6000,0	0	0	0

1	2	3	4	5	6	7
19.	Consolidating the State Public Health Surveillance Service and other health services at the national and territorial levels for ensuring the fulfillment of essential public health operations and services.	185,0	185,0	0	0	0
20.	Strengthening the technical-material base in compliance with the modern requirements for scientific research, including developing new research methods and innovation technologies	5000,0	2500,0	250,0	2000,0	250,0
21.	Developing and using efficiently the scientific and technological potential	1000,0	600,0	0	200,0	200,0
22.	Extending collaboration scientific relations by establishing partnerships with domestic and overseas scientific research centers and institutions for carrying out scientific research in public health area	500,0	200,0	0	200,0	100,0
Total		93850,5	22380,5	12700,0	55630,0	3140,0
Year 2016						
1.	Enhancing and increasing the efficiency of the surveillance system for diseases and population health indicators.	1600,0	950,0	300,0	350,0	0
2.	Monitoring and periodically evaluating population health status, communicating the obtained results to interested authorities and population	600,0	200,0	0	400,0	0
3.	Strengthening the national capacities for early detection and rapid response to public health threats and emergencies based on risk monitoring indicators at local, intermediary (district/municipality) and national levels	1450,0	1100,0	0	350,0	0
4.	Minimizing the negative impact on population health, determined by public health risks and emergencies caused by natural, technogenic, biological, radiological and social factors	700,0	400,0	0	100,0	200,0
5.	Improving the legal and regulatory framework in health protection area by harmonizing it with the international regulations	260,0	130,0	0	130,0	0
6.	Enhancing the State Public Health Surveillance Service capacities to assess, manage, and communicate the risks for public health and to establish the mechanism for collaboration with other services in the area	12650,0	0	3000,0	9650,0	0
7.	Consolidating the health sector's infrastructure and capacities in health promotion area	1640,0	40,0	1330,0	250,0	20,0
8.	Increasing the potential and involvement of other sectors than health sector in activities for health promotion and improvement of inter-sector collaboration at the national, and territorial/local levels	1250,0	100,0	1150,0	0	0

1	2	3	4	5	6	7
9.	Creating health environments by establishing partnerships with civil society and empowering communities to plan and implement efficient health promotions activities	1400,0	300,0	500,0	600,0	0
10.	Decreasing the risk factors at the individual, community, and national levels through integrated activities of health sector together with other sectors and local public authorities	150,0	60,0	90,0	0	0
11.	Preventing and controlling vaccine-preventable diseases by covering the target groups with vaccines in line with the provisions of the national immunization programs and to decrease communicable diseases' morbidity by 2% by 2020 via measures of primary and secondary prevention	50,0	30,0	0	20,0	0
12.	Ensuring equitable access to quality services of disease prevention provided by primary, hospital, and specialized health care and public health surveillance service	1000,0	0	700,0	300,0	0
13.	Strengthening collaboration between the health sector and other sectors so as to reduce the risk factors and improve population health status, applying the "health in all policies" principle	40,0	10,0	5,0	15,0	10,0
14.	Improving the quality of medical and public health services, efficiency of interventions to reduce inequity in health	50,0	15,0	0	35,0	0
15.	Implementing e-governance mechanisms in public health services	290,0	140,0	10,0	90,0	50,0
16.	Staffing the public health system structures with sufficient number of qualified personnel and to enhance the legislative framework; to estimate the needs and to plan human resources for carrying out public health essential operations	130,0	40,0	0	90,0	0
17.	Modernizing the process and stages of professional training, adjusting the curricular components of the pre-university, undergraduate, postgraduate, and continuous training of the personnel in line with the essential competences for the public health set by the Association of Schools of Public Health in the European Region (ASPHER)	100,0	90,0	0	10,0	0
18.	Staffing the public health system with human resources in a sufficient and balanced way by developing and implementing innovating programs for recruiting, maintaining, and developing the personnel	6000,0	6000,0	0	0	0
19.	Consolidating the State Public Health Surveillance Service and other health services at the national and territorial levels for ensuring the fulfillment of essential public health operations and services.	13575,0	13575,0	0	0	0

1	2	3	4	5	6	7
20.	Strengthening the technical-material base in compliance with the modern requirements for scientific research, including developing new research methods and innovation technologies	5000,0	2500,0	250,0	2000,0	250,0
21.	Developing and using efficiently the scientific and technological potential	1000,0	600,0	0	200,0	200,0
22.	Extending collaboration scientific relations by establishing partnerships with domestic and overseas scientific research centers and institutions for carrying out scientific research in public health area	500,0	200,0	0	200,0	100,0
Total		49435,0	26480,0	7335,0	14790,0	830,0
Year 2017						
1.	Enhancing and increasing the efficiency of the surveillance system for diseases and population health indicators.	2900,0	1950,0	500,0	450,0	0
2.	Monitoring and periodically evaluating population health status, communicating the obtained results to interested authorities and population	600,0	200,0	0	400,0	0
3.	Strengthening the national capacities for early detection and rapid response to public health threats and emergencies based on risk monitoring indicators at local, intermediary (district/municipality) and national levels	950,0	700,0	0	250,0	0
4.	Minimizing the negative impact on population health, determined by public health risks and emergencies caused by natural, technogenic, biological, radiological and social factors	200,0	50,0	0	100,0	50,0
5.	Improving the legal and regulatory framework in health protection area by harmonizing it with the international regulations	160,0	80,0	0	80,0	0
6.	Enhancing the State Public Health Surveillance Service capacities to assess, manage, and communicate the risks for public health and to establish the mechanism for collaboration with other services in the area	150,0	0	0	150,0	0
7.	Consolidating the health sector's infrastructure and capacities in health promotion area	740,0	70,0	420,0	250,0	0
8.	Increasing the potential and involvement of other sectors than health sector in activities for health promotion and improvement of inter-sector collaboration at the national, and territorial/local levels	1250,0	150,0	1100,0	0	0
9.	Creating health environments by establishing partnerships with civil society and empowering communities to plan and implement efficient health promotions activities	1400,0	600,0	400,0	400,0	0

1	2	3	4	5	6	7
10.	Decreasing the risk factors at the individual, community, and national levels through integrated activities of health sector together with other sectors and local public authorities	150,0	70,0	80,0	0	0
11.	Preventing and controlling vaccine-preventable diseases by covering the target groups with vaccines in line with the provisions of the national immunization programs and to decrease communicable diseases' morbidity by 2% by 2020 via measures of primary and secondary prevention	50,0	20,0	0	30,0	0
12.	Ensuring equitable access to quality services of disease prevention provided by primary, hospital, and specialized health care and public health surveillance service	1000,0	0	700,0	300,0	0
13.	Strengthening collaboration between the health sector and other sectors so as to reduce the risk factors and improve population health status, applying the "health in all policies" principle	40,0	15,0	5,0	10,0	10,0
14.	Improving the quality of medical and public health services, efficiency of interventions to reduce inequity in health	0	0	0	0	0
15.	Implementing e-governance mechanisms in public health services	40	20,0	10,0	0	10,0
16.	Staffing the public health system structures with sufficient number of qualified personnel and to enhance the legislative framework; to estimate the needs and to plan human resources for carrying out public health essential operations	100, 0	40,0	0	60,0	0
17.	Modernizing the process and stages of professional training, adjusting the curricular components of the pre-university, undergraduate, postgraduate, and continuous training of the personnel in line with the essential competences for the public health set by the Association of Schools of Public Health in the European Region (ASPHER)	100,0	90,0	0	10,0	0
18.	Staffing the public health system with human resources in a sufficient and balanced way by developing and implementing innovating programs for recruiting, maintaining, and developing the personnel	6000,0,0	6000,0	0	0	0
19.	Consolidating the State Public Health Surveillance Service and other health services at the national and territorial levels for ensuring the fulfillment of essential public health operations and services.	75,0	75,0	0	0	0
20.	Strengthening the technical-material base in compliance with the modern requirements for scientific research, including developing new research methods and innovation technologies	2000,0	800,0	100,0	1000,0	100,0
21.	Developing and using efficiently the scientific and technological potential	500,0	300,0	0	100,0	100,0

1	2	3	4	5	6	7
22.	Extending collaboration scientific relations by establishing partnerships with domestic and overseas scientific research centers and institutions for carrying out scientific research in public health area	300,0	150,0	0	100,0	50,0
Total		18705,0	11380,0	3315,0	3690,0	320,0
Year 2018						
1.	Enhancing and increasing the efficiency of the surveillance system for diseases and population health indicators.	3400,0	2700,0	400,0	300,0	0
2.	Monitoring and periodically evaluating population health status, communicating the obtained results to interested authorities and population	600,0	300,0	0	300,0	0
3.	Strengthening the national capacities for early detection and rapid response to public health threats and emergencies based on risk monitoring indicators at local, intermediary (district/municipality) and national levels	1350,0	1100,0	0	250,0	0
4.	Minimizing the negative impact on population health, determined by public health risks and emergencies caused by natural, technogenic, biological, radiological and social factors	200,0	80,0	0	100,0	20,0
5.	Improving the legal and regulatory framework in health protection area by harmonizing it with the international regulations	60,0	0	0	60,0	0
6.	Enhancing the State Public Health Surveillance Service capacities to assess, manage, and communicate the risks for public health and to establish the mechanism for collaboration with other services in the area	100,0	0	0	100,0	0
7.	Consolidating the health sector's infrastructure and capacities in health promotion area	740,0	40,0	500,0	200,0	0
8.	Increasing the potential and involvement of other sectors than health sector in activities for health promotion and improvement of inter-sector collaboration at the national, and territorial/local levels	1250,0	125,0	1125,0	0	0
9.	Creating health environments by establishing partnerships with civil society and empowering communities to plan and implement efficient health promotions activities	900,0	200,0	300,0	400,0	0
10.	Decreasing the risk factors at the individual, community, and national levels through integrated activities of health sector together with other sectors and local public authorities	150,0	70,0	80,0	0	0
11.	Preventing and controlling vaccine-preventable diseases by covering the target groups with vaccines in line with the provisions of the national immunization programs and to decrease communicable diseases' morbidity by 2% by 2020 via measures of primary and secondary prevention	250,0	100,0	0	150,0	0

1	2	3	4	5	6	7
12.	Ensuring equitable access to quality services of disease prevention provided by primary, hospital, and specialized health care and public health surveillance service	1000,0	0	700,0	300,0	0
13.	Strengthening collaboration between the health sector and other sectors so as to reduce the risk factors and improve population health status, applying the "health in all policies" principle	40,0	19,0	5,0	10,0	6,0
14.	Improving the quality of medical and public health services, efficiency of interventions to reduce inequity in health	0	0	0	0	0
15.	Implementing e-governance mechanisms in public health services	40,0	20,0	10,0	0	10,0
16.	Staffing the public health system structures with sufficient number of qualified personnel and to enhance the legislative framework; to estimate the needs and to plan human resources for carrying out public health essential operations	0	0	0	0	0
17.	Modernizing the process and stages of professional training, adjusting the curricular components of the pre-university, undergraduate, postgraduate, and continuous training of the personnel in line with the essential competences for the public health set by the Association of Schools of Public Health in the European Region (ASPHER)	150,0	120,0	0	30,0	0
18.	Staffing the public health system with human resources in a sufficient and balanced way by developing and implementing innovating programs for recruiting, maintaining, and developing the personnel	6000,0	6000,0	0	0	0
19.	Consolidating the State Public Health Surveillance Service and other health services at the national and territorial levels for ensuring the fulfillment of essential public health operations and services.	60,0	60,0	0	0	0
20.	Strengthening the technical-material base in compliance with the modern requirements for scientific research, including developing new research methods and innovation technologies	1000,0	400,0	50,0	500,0	50,0
21.	Developing and using efficiently the scientific and technological potential	500,0	300,0	0	100,0	100,0
22.	Extending collaboration scientific relations by establishing partnerships with domestic and overseas scientific research centers and institutions for carrying out scientific research in public health area	300,0	150,0	0	100,0	50,0
Total		18090,0	11784,0	3170,0	2900,0	236,0
Year 2019						
1.	Enhancing and increasing the efficiency of the surveillance system for diseases and population health indicators.	650,0	250,0	100,0	300,0	0

1	2	3	4	5	6	7
2.	Monitoring and periodically evaluating population health status, communicating the obtained results to interested authorities and population	600,0	300,0	0	300,0	0
3.	Strengthening the national capacities for early detection and rapid response to public health threats and emergencies based on risk monitoring indicators at local, intermediary (district/municipality) and national levels	1350,0	1050,0	0	300,0	0
4.	Minimizing the negative impact on population health, determined by public health risks and emergencies caused by natural, technogenic, biological, radiological and social factors	200,0	100,0	0	100,0	0
5.	Improving the legal and regulatory framework in health protection area by harmonizing it with the international regulations	0	0	0	0	0
6.	Enhancing the State Public Health Surveillance Service capacities to assess, manage, and communicate the risks for public health and to establish the mechanism for collaboration with other services in the area	100,0	0	0	100,0	0
7.	Consolidating the health sector's infrastructure and capacities in health promotion area	690,0	40,0	500,0	150,0	0
8.	Increasing the potential and involvement of other sectors than health sector in activities for health promotion and improvement of inter-sector collaboration at the national, and territorial/local levels	0	0	0	0	0
9.	Creating health environments by establishing partnerships with civil society and empowering communities to plan and implement efficient health promotions activities	500,0	250,0	150,0	100,0	0
10.	Decreasing the risk factors at the individual, community, and national levels through integrated activities of health sector together with other sectors and local public authorities	150,0	70,0	80,0	0	0
11.	Preventing and controlling vaccine-preventable diseases by covering the target groups with vaccines in line with the provisions of the national immunization programs and to decrease communicable diseases' morbidity by 2% by 2020 via measures of primary and secondary prevention	50,0	30,0	0	20,0	0
12.	Ensuring equitable access to quality services of disease prevention provided by primary, hospital, and specialized health care and public health surveillance service	1000,0	0	700,0	300,0	0
13.	Strengthening collaboration between the health sector and other sectors so as to reduce the risk factors and improve population health status, applying the "health in all policies" principle	0	0	0	0	0

1	2	3	4	5	6	7
14.	Improving the quality of medical and public health services, efficiency of interventions to reduce inequity in health	0	0	0	0	0
15.	Implementing e-governance mechanisms in public health services	40,0	20,0	10,0	0	10,0
16.	Staffing the public health system structures with sufficient number of qualified personnel and to enhance the legislative framework; to estimate the needs and to plan human resources for carrying out public health essential operations	0	0	0	0	0
17.	Modernizing the process and stages of professional training, adjusting the curricular components of the pre-university, undergraduate, postgraduate, and continuous training of the personnel in line with the essential competences for the public health set by the Association of Schools of Public Health in the European Region (ASPHER)	250,0	180,0	0	70,0	0
18.	Staffing the public health system with human resources in a sufficient and balanced way by developing and implementing innovating programs for recruiting, maintaining, and developing the personnel	6000,0	6000,0	0	0	0
19.	Consolidating the State Public Health Surveillance Service and other health services at the national and territorial levels for ensuring the fulfillment of essential public health operations and services.	60,0	60,0	0	0	0
20.	Strengthening the technical-material base in compliance with the modern requirements for scientific research, including developing new research methods and innovation technologies	1000,0	400,0	50,0	500,0	50,0
21.	Developing and using efficiently the scientific and technological potential	500,0	300,0	0	100,0	100,0
22.	Extending collaboration scientific relations by establishing partnerships with domestic and overseas scientific research centers and institutions for carrying out scientific research in public health area	200,0	50,0	0	100,0	50,0
Total		13340,0	9100,0	1590,0	2440,0	210,0
Year 2020						
1.	Enhancing and increasing the efficiency of the surveillance system for diseases and population health indicators.	1650,0	1050,0	300,0	300,0	0
2.	Monitoring and periodically evaluating population health status, communicating the obtained results to interested authorities and population	100,0	0	0	100,0	0

1	2	3	4	5	6	7
3.	Strengthening the national capacities for early detection and rapid response to public health threats and emergencies based on risk monitoring indicators at local, intermediary (district/municipality) and national levels	1350,0	1050,0	0	300,0	0
4.	Minimizing the negative impact on population health, determined by public health risks and emergencies caused by natural, technogenic, biological, radiological and social factors	200,0	70,0	0	100,0	30,0
5.	Improving the legal and regulatory framework in health protection area by harmonizing it with the international regulations	0	0	0	0	0
6.	Enhancing the State Public Health Surveillance Service capacities to assess, manage, and communicate the risks for public health and to establish the mechanism for collaboration with other services in the area	100,0	0	0	100,0	0
7.	Consolidating the health sector's infrastructure and capacities in health promotion area	690,0	140,0	500,0	50,0	0
8.	Increasing the potential and involvement of other sectors than health sector in activities for health promotion and improvement of inter-sector collaboration at the national, and territorial/local levels	0	0	0	0	0
9.	Creating health environments by establishing partnerships with civil society and empowering communities to plan and implement efficient health promotions activities	500,0	250,0	150,0	100,0	0
10.	Decreasing the risk factors at the individual, community, and national levels through integrated activities of health sector together with other sectors and local public authorities	150,0	70,0	80,0	0	0
11.	Preventing and controlling vaccine-preventable diseases by covering the target groups with vaccines in line with the provisions of the national immunization programs and to decrease communicable diseases' morbidity by 2% by 2020 via measures of primary and secondary prevention	0	0	0	0	0
12.	Ensuring equitable access to quality services of disease prevention provided by primary, hospital, and specialized health care and public health surveillance service	1000,0	0	700,0	300,0	0
13.	Strengthening collaboration between the health sector and other sectors so as to reduce the risk factors and improve population health status, applying the "health in all policies" principle	0	0	0	0	0
14.	Improving the quality of medical and public health services, efficiency of interventions to reduce inequity in health	0	0	0	0	0
15.	Implementing e-governance mechanisms in public health services	40,0	20,0	10,0	0	10,0

1	2	3	4	5	6	7
16.	Staffing the public health system structures with sufficient number of qualified personnel and to enhance the legislative framework; to estimate the needs and to plan human resources for carrying out public health essential operations	0	0	0	0	0
17.	Modernizing the process and stages of professional training, adjusting the curricular components of the pre-university, undergraduate, postgraduate, and continuous training of the personnel in line with the essential competences for the public health set by the Association of Schools of Public Health in the European Region (ASPHER)	300,0	220,0	0	80,0	0
18.	Staffing the public health system with human resources in a sufficient and balanced way by developing and implementing innovating programs for recruiting, maintaining, and developing the personnel	6000,0	6000,0	0	0	0
19.	Consolidating the State Public Health Surveillance Service and other health services at the national and territorial levels for ensuring the fulfillment of essential public health operations and services.	60,0	60,0	0	0	0
20.	Strengthening the technical-material base in compliance with the modern requirements for scientific research, including developing new research methods and innovation technologies	1000,0	900,0	50,0	0	50,0
21.	Developing and using efficiently the scientific and technological potential	500,0	300,0	0	100,0	100,0
22.	Extending collaboration scientific relations by establishing partnerships with domestic and overseas scientific research centers and institutions for carrying out scientific research in public health area	200,0	50,0	0	100,0	50,0
Total		13840,0	10180,0,0	1790,0	1630,0	240,0
TOTAL, years 2014 – 2020						
1.	Enhancing and increasing the efficiency of the surveillance system for diseases and population health indicators.	11850.0	7400.0	2000.0	2450.0	0
2.	Monitoring and periodically evaluating population health status, communicating the obtained results to interested authorities and population	3700.0	1500.0	0	2200.0	0
3.	Strengthening the national capacities for early detection and rapid response to public health threats and emergencies based on risk monitoring indicators at local, intermediary (district/municipality) and national levels	8380,0	6280,0	0	2100,0	0
4.	Minimizing the negative impact on population health, determined by public health risks and emergencies caused by natural, technogenic, biological, radiological and social factors	1500,0	700,0	0	500,0	300,0

1	2	3	4	5	6	7
5.	Improving the legal and regulatory framework in health protection area by harmonizing it with the international regulations	1000,0	450,0	0	550,0	0
6.	Enhancing the State Public Health Surveillance Service capacities to assess, manage, and communicate the risks for public health and to establish the mechanism for collaboration with other services in the area	136600,0	11825,0	7000,0	117775,0	0
7.	Consolidating the health sector's infrastructure and capacities in health promotion area	43400,0	700,0	41100,0	1500,0	100,0
8.	Increasing the potential and involvement of other sectors than health sector in activities for health promotion and improvement of inter-sector collaboration at the national, and territorial/local levels	5000,0	500,0	4500,0	0	0
9.	Creating health environments by establishing partnerships with civil society and empowering communities to plan and implement efficient health promotions activities	7000,0	2000,0	2000,0	3000,0	0
10.	Decreasing the risk factors at the individual, community, and national levels through integrated activities of health sector together with other sectors and local public authorities	1150,0	550,0	600,0	0	0
11.	Preventing and controlling vaccine-preventable diseases by covering the target groups with vaccines in line with the provisions of the national immunization programs and to decrease communicable diseases' morbidity by 2% by 2020 via measures of primary and secondary prevention	700,0	300,0	0	400,0	0
12.	Ensuring equitable access to quality services of disease prevention provided by primary, hospital, and specialized health care and public health surveillance service	9000,0	0	7000,0	2000,0	0
13.	Strengthening collaboration between the health sector and other sectors so as to reduce the risk factors and improve population health status, applying the "health in all policies" principle	2526,0	405,0	100,0	95,0	1926,0
14.	Improving the quality of medical and public health services, efficiency of interventions to reduce inequity in health	139,0	39,0	0	100,0	0
15.	Implementing e-governance mechanisms in public health services	12920,0	2330,0	6350,0	740,0	3500,0
16.	Staffing the public health system structures with sufficient number of qualified personnel and to enhance the legislative framework; to estimate the needs and to plan human resources for carrying out public health essential operations	450,0	200,0	0	250,0	0

1	2	3	4	5	6	7
17.	Modernizing the process and stages of professional training, adjusting the curricular components of the pre-university, undergraduate, postgraduate, and continuous training of the personnel in line with the essential competences for the public health set by the Association of Schools of Public Health in the European Region (ASPHER)	900,0	700,0	0	200,0	0
18.	Staffing the public health system with human resources in a sufficient and balanced way by developing and implementing innovating programs for recruiting, maintaining, and developing the personnel	42000,0	42000,0	0	0	0
19.	Consolidating the State Public Health Surveillance Service and other health services at the national and territorial levels for ensuring the fulfillment of essential public health operations and services.	14135,0	14135,0	0	0	0
20.	Strengthening the technical-material base in compliance with the modern requirements for scientific research, including developing new research methods and innovation technologies	20000,0	10000,0	1000,0	8000,0	1000,0
21.	Developing and using efficiently the scientific and technological potential	5000,0	3000,0	0	1000,0	1000,0
22.	Extending collaboration scientific relations by establishing partnerships with domestic and overseas scientific research centers and institutions for carrying out scientific research in public health area	2500,0	1000,0	0	1000,0	500,0
Total		329850,0	106014,0	71650,0	143860,0	8326,0

ANEX NO. 3

OF THE NATIONAL PUBLIC HEALTH STRATEGY FOR 2014-2020

Expected results and progress indicators of the National Public Health Strategy for 2014-2020

No.	Specific objectives	Name of the indicator	Evaluation period	
			2016	2020
1	2	3	4	5
1.	To make more efficient and to consolidate the population health surveillance systems so as to identify the health problems and to supply relevant, accurate, and timely information for decisions and actions in public health area.	1. Number of implemented and operational surveillance systems (communicable diseases and events of public health, nutritional health, occupational health, environmental health, healthcare-associated infections and antimicrobial resistance) 2. Share of medical-sanitary institutions applying the data quality management system, % 3. Comparability rate of the national public health indicators with the European Community indicators, %	2 10 10	6 50 50
2.	To strengthen the national system for prevention, preparedness and response in public health emergencies through integrated approach to threats	1. Share of medical-sanitary institutions, which implemented the system of early warning and rapid response mechanism, % 2. Share of public health events of biological, chemical, and radiological nature, which were investigated by the laboratories of the State Public Health Surveillance Service 3. Share of authorities involved in the early warning and rapid response system for prevention and control of communicable diseases and public health events with adjusted and approved operational procedures, %	15 40 15	45 60 45

1	2	3	4	5
3.	To ensure health protection by making more efficient the control over behavioral and environmental risk factors	1. Number of national policy documents and actions for improving the environmental factors and reducing the risks for health 2. Approval of the restrictions for using trans fatty acids and salt in the processed food-stuff	2 -	5 +
4.	To adopt some healthy behaviors among the population by implementing efficient and coordinate measures for health promotion in the different sectors at the national and local levels	1. Share of population from the target groups covered by the program for early identification of priority non-communicable diseases and counselling, % 2. Share of adult population practicing active lifestyle (at least 150 min. of medium-intensity activity per week), % 3. Alcohol consumption among adults (15 years and over) in liters per inhabitant per year 4. Share of smoking pregnant women, %	50 40 17,0 5	80 50 16,0 4
5.	To reduce the burden of communicable and non-communicable diseases by reducing the risk factors and ensuring equitable access of population to primary, secondary, and tertiary prevention services	1. Share of vaccination coverage in the target groups according to the programs' provisions, % 2. Total morbidity of infectious diseases with acute diarrhea syndrome per 100 thousand inhabitants 3. Incidence of measles per 100 thousand inhabitants 4. Prevalence of anemia among pregnant women, % 5. Prevalence of persons with high blood pressure (systolic ≥ 140 and or diastolic ≥ 90 mmHg), %	93 514,3 0,05 30 40	95 437,2 0,01 20 30
6.	To improve the cross-sector collaboration mechanisms with clear definition of responsibilities among the health sector and other sectors in implementing the essential public health operations	1. Share of local public authorities of I level involved in implementing the guides and actions for health promotion, % 2. Share of local public authorities, which implement at the territorial level different local public health programs, %	30 60	60 80
7.	To ensure with sufficient and competent human resources in public health area by strengthening the training system for undertaking the essential public health operations	1. Share of coverage with human resources of the State Public Health Surveillance Service, % 2. Share of specialists working in public health areas and having relevant skills and knowledge, %	70 80	90 100

1	2	3	4	5
8.	To adjust the organizational structure and to improve the financing of the State Public Health Surveillance Service so as to carry out the essential operations and services in public health area in collaboration with other sectors	1. Share of public health services' financing from the global budget for health, % 2. Share granted from the prevention fund of the National Health Insurance Company for carrying out health programs focused on prevention, %	3 30	5 50
9.	To consolidate research in the area of public health so as to ensure evidence-based reasoning for policies	1. Number of research projects in the area of public health with implemented results/innovations, used as evidence for developing the framework for reducing mortality, morbidity, disability due to communicable and non-communicable diseases	10/3	20/4

